



Accreditation Commission for
Acupuncture and Herbal Medicine

Comprehensive Standards and Criteria

Reference Copy – February 2026

ACAHM makes available a combined reference copy of the Comprehensive Standards and Criteria for Accreditation (“Comprehensive Standards”) for reading convenience. The individual Introduction, Standards and Revision History documents posted at the ACAHM website represent the official publication of the Comprehensive Standards. While the Commission makes suitable effort to assure that the reference copy reflects the contents of the official publication, the reference copy is not warranted as such.

Always be sure that you are working from a current version of the Comprehensive Standards.

INTRODUCTION TO THE ACAHM COMPREHENSIVE STANDARDS AND CRITERIA

The Comprehensive Standards and Criteria of the Accreditation Commission for Acupuncture and Herbal Medicine (ACAHM or Commission) are the written benchmarks, conditions, and thresholds adopted by the Commission to: (1) assess whether the quality of an institution/program merits pre-accreditation/accreditation status, and (2) maintain a culture of continuous improvement of academic quality at the institutions/programs.

There are ten (10) standards that serve as broad classifications used to categorize related accreditation criteria. Many criteria contain sub-components relating to the institution or each ACAHM-accreditable program.

All revisions to the comprehensive standards and criteria and their chronology are recorded in the [Revision History](#).

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ACAHM-ACCREDITABLE PROGRAMS (see [Glossary](#) and [Position Paper: ACAHM-accreditable Doctoral Programs](#))

Only programs for which ACAHM has established accreditation standards and criteria are accreditable. These Comprehensive Standards and Criteria apply to the following programs for which ACAHM currently awards accredited or pre-accredited status:

- **Master’s-level degree and diploma programs**
- **Professional entry-level doctoral degree programs**
- **Advanced practice doctoral degree program**
- **Graduate certificate programs**

ACUPUNCTURE AND HERBAL MEDICINE DEFINITIONS (see [Glossary](#))

As defined by ACAHM’s *Glossary*, “Acupuncture and Herbal Medicine (AHM)” is a collective term for systems of medicine, incorporating fundamental principles, physiological concepts, theoretical foundations, diagnostic procedures, and treatment modalities, whose fundamental theory originated in China and further developed in a variety of other predominantly Asian cultures. Treatment modalities include acupuncture, herbal medicine, manual therapy, diet/lifestyle counseling, and therapeutic exercise.

The term “acupuncture” can be used to describe a specific modality or a comprehensive system of medicine.

Modality – The practice of “acupuncture” includes any insertion and/or manipulation of a filiform acupuncture needle for a therapeutic purpose based upon both traditional theoretical understanding and modern scientific study of the musculoskeletal and nervous system. This practice encompasses interventions described by terminology such as “dry needling,” “motor point needling,” and “myofascial trigger point needling.”

“Acupuncture” as a modality is distinguished from the use of the term to generally describe the system of medicine also referred to as Acupuncture and Herbal Medicine, East Asian medicine, traditional Chinese medicine, Oriental medicine, etc., which includes a variety of techniques or therapeutic approaches, including acupuncture, for the promotion and maintenance of health and prevention of disease.

The Commission uses capitalization of “Acupuncture” and “Acupuncture and Herbal Medicine” to differentiate the system of medicine from the individual treatment modalities.

DISTANCE EDUCATION (see [Glossary](#) and [Distance Education Policy](#))

All distance education must comply with ACAHM’s Comprehensive Standards and Criteria for Accreditation.

INSTITUTIONAL vs. PROGRAMMATIC CRITERIA/COMPONENTS

ACAHM is recognized by the United States Department of Education as both an institutional accreditor and as a programmatic accreditor. Components of the accreditation criteria are differentiated into two types, institutional and programmatic. Criteria may include one or both types of components.

- **Institutional Components:** These components are defined as those relevant only to entities that are institutionally accredited by ACAHM. These institutional components are not relevant or pertinent to the schools with other institutional accreditors that rely on ACAHM for programmatic accreditation only.
- **Programmatic Components:** These components are defined as those that are relevant to ACAHM programs, regardless of the institutional accreditation. Programmatic components may be further differentiated as applicable to all programs (i.e., master’s, professional doctoral, advanced practice doctoral) or applicable to only one program.

ELIGIBILITY REQUIREMENTS (see [Glossary](#))

Eligibility Requirements are select accreditation criteria further classified by the Commission as ACAHM's basic threshold requirements for institutions, master's-level programs, and doctoral programs to achieve and maintain ACAHM pre-accreditation and accreditation status. An institution/program seeking pre-accreditation must demonstrate that it complies with these select accreditation criteria further classified by the Commission as *Eligibility Requirements*. While all accreditation standards and criteria are assessed during the eligibility phase, compliance with the following criteria designated as *Eligibility Requirements* is a requirement for achieving pre-accreditation status:

<i>Standard</i>	<i>Criterion</i>	<i>Title</i>
1	1.01	Integrity
1	1.02	Mission and Statement of Purpose
1	1.03	Program Goals
1	1.04	Program Learning Outcomes
2	2.01	Legal Organization and Authorization
2	2.02	Accreditation Status of Parent Institution
2	2.03	Degree-Granting Authority
2	2.05	Compliance with Federal Law
2	2.06	Compliance with State Law
2	2.07	Compliance with Local and Municipal Laws, Ordinances and Codes
3	3.01	Governance Structure
3	3.05	Administrative & Academic Leadership
5	5.01	Recruitment and Policy Publication
5	5.02	Admissions
5	5.05	Student Services
6	6.01	Systematic Evaluation
6	6.02	Assessment of Student Learning
7	7.01	Program Level
7	7.02	Minimum Program Length, Credits, and Hours
8	8.01	Core Faculty
9	9.02	General Sufficiency and Stability of Resources
9	9.04	Financial Planning and Control
9	9.08	Campus and Program Facilities
9	9.09	Learning Resource System (Library)
10	10.02	Catalog

PUBLICATION AND FORMATTING

The ACAHM Comprehensive Standards and Criteria consist of twelve (12) documents:

- Introduction to the Comprehensive Standards and Criteria
- Ten (10) Standards documents
- Comprehensive Standards and Criteria Revision History

A unified document containing all twelve (12) documents is available for user convenience but does not constitute the official published Comprehensive Standards and Criteria.

Within individual Standards and Criteria, the following section and sub-section format is employed:

- A. ____
 - 1. ____
 - a) ____

The letters I, O and Q are intentionally omitted from the format, due to their close visual similarity to the numbers 1 and 0.

PUBLICATIONS, RECORDS, AND MISCELLANEOUS DOCUMENTATION

When a criterion for accreditation requires published material, the institution/program must retain a copy of said material in English, regardless of the language intended for distribution.

For the purposes of accreditation procedures, the institution/program must be able to produce copies of all published material in English, regardless of the language intended for distribution.

Published materials cited in the accreditation standards and criteria include, but are not limited to, promotional advertisements, website, catalog, clinic manual, student handbook, enrollment agreements, syllabi, academic records, and required regulatory disclosures.

RELATED POLICIES/RESOURCES

Programs and institutions seeking or holding accreditation status with the Accreditation Commission for Acupuncture and Herbal Medicine (ACAHM or Commission) are subject to the Commission's *Standards, Criteria, Policies and Procedures* and all associated *Applications, Checklists, Glossaries, Guides, Handbooks, Manuals* and *Position Papers* published by the Commission on its website. Specific policies/resources may be hyperlinked within the published Standards documents.

SITE VISIT MANUAL

Programs and institutions demonstrate their compliance with the Comprehensive Standards and Criteria through the self-study process, which includes preparation of a self-study report, peer review of the report, and a site visit by peer evaluators. Consult the [ACAHM Site Visit Manual](#) for examples of the kinds of questions that programs and institutions should seek to answer through the self-study process, and the types of exhibits and resources that help to demonstrate compliance with the Comprehensive Standards and Criteria.

TRANSITIONS TO NEW/REVISED STANDARDS/CRITERIA

When ACAHM implements new or revised accreditation standards/criteria, institutions/programs are encouraged to move toward compliance with the most current accreditation standards. However, for decision-making purposes, ACAHM will hold institutions/programs accountable to meeting the standards under which the most recent accreditation application was submitted. Institutions/programs may request accountability to meet more current standards subject to Commission approval.

ACAHM COMPREHENSIVE STANDARDS AND CRITERIA

STANDARD 1: PURPOSE

Criterion 1.01: INTEGRITY	1
Criterion 1.02: MISSION AND STATEMENT OF PURPOSE.....	1
Criterion 1.03: PROGRAM GOALS	2
Criterion 1.04: PROGRAM LEARNING OUTCOMES.....	3
Criterion 1.05: RESOURCE ALLOCATION	3
Criterion 1.06: REVIEW	4

Criterion 1.01: INTEGRITY	
Related Policies:	Integrity in the Accreditation Process Policy
References:	

INSTITUTIONAL COMPONENTS

The institution must conduct its operations with honesty and integrity.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program must conduct its operations with honesty and integrity.

Criterion 1.02: MISSION AND STATEMENT OF PURPOSE	
Related Policies:	
References:	

INSTITUTIONAL COMPONENTS

The institution must develop and maintain a formally adopted mission statement and strategic plan that provides clear direction and vision for the institution and its programs.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

In accordance with the institution's mission, each program must develop and maintain a separate and distinct formally adopted educational statement of purpose that provides clear direction for the program.

Criterion 1.03: PROGRAM GOALS	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. Each program must enumerate a set of broadly defined goals. Each goal must support the professional development of students.

PROFESSIONAL ENTRY-LEVEL DOCTORAL PROGRAMS

- B. The entry-level doctoral program's goals must address the following general principles:
1. Providing the student with an education in core didactic and core clinical areas, with an emphasis on clinical assessment, diagnosis, and intervention (broadly defined to include treatment and treatment planning).
 2. Developing the student's knowledge and understanding of how to engage in collaborative interactions involving Acupuncture and Herbal Medicine (AHM)¹ practitioners and other health care practitioners.
 3. Developing the student's ability to think critically, synthesize knowledge, and use research findings along with scientific and scholarly literature to support their professional practice.

ADVANCED PRACTICE DOCTORAL PROGRAMS

- C. The advanced practice doctoral program's goals must address the following general principles:
1. Providing the student with an advanced, in-depth education in core didactic, core clinical, specialty didactic, and specialty clinical areas, with an emphasis on clinical assessment, diagnosis, and intervention (broadly defined to include treatment and treatment planning).
 2. Advancing the student's ability to apply AHM-related modalities.
 3. Advancing the student's ability to effectively engage in collaborative interactions involving AHM practitioners and other health care practitioners.
 4. Advancing the student's ability to think critically, synthesize knowledge, and use research findings along with scientific and scholarly literature to support their professional practice.

¹ As defined by ACAHM's *Glossary*, "Acupuncture and Herbal Medicine (AHM)" is a collective term for systems of medicine, incorporating fundamental principles, physiological concepts, theoretical foundations, diagnostic procedures, and treatment modalities, whose fundamental theory originated in China and further developed in a variety of other predominantly Asian cultures. Treatment modalities include acupuncture, herbal medicine, manual therapy, diet/lifestyle counseling, and therapeutic exercise. The Commission uses capitalization of "Acupuncture and Herbal Medicine" to differentiate the system of medicine from the individual treatment modalities.

5. Cultivating the student's ability to contribute to AHM-related scholarly literature.
6. Providing the student with an opportunity for advanced discourse between faculty and students which results in the development of an academic community that will enrich and advance the profession by contributing to the development of future generations of practitioners, faculty, researchers, clinical supervisors, and leaders of the profession.

Criterion 1.04: PROGRAM LEARNING OUTCOMES	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. Each program must establish and maintain a set of clearly specified and measurable program learning outcomes.
- B. Program learning outcomes must be consistent with the program's statement of purpose and its goals.
- C. The program learning outcomes must lead to the development of the relevant professional competencies described in *Criterion 7.04*.

Criterion 1.05: RESOURCE ALLOCATION	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

The institution's current or projected academic programs, services, activities, resources, and resource allocation decisions must be consistent with its mission statement.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program's resources, and its current or projected services and activities, must be consistent with its statement of purpose, goals, and learning outcomes.

Criterion 1.06: REVIEW	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

The institution's mission statement must be reviewed periodically by the institution's stakeholders and communities of interest. The mission statement must be revised when necessary to ensure its continued relevance and accuracy.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

Each program's statement of purpose, goals, and learning outcomes must be reviewed periodically by stakeholders and communities of interest. The program's statement of purpose, goals, and learning outcomes must be revised when necessary to ensure their continued relevance and accuracy.

ACAHM COMPREHENSIVE STANDARDS AND CRITERIA

STANDARD 2: LEGAL AND REGULATORY REQUIREMENTS

Criterion 2.01: LEGAL ORGANIZATION AND AUTHORIZATION	1
Criterion 2.02: ACCREDITATION STATUS OF PARENT INSTITUTION	2
Criterion 2.03: DEGREE-GRANTING AUTHORITY	2
Criterion 2.04: OFF-CAMPUS CONTROL.....	2
Criterion 2.05: COMPLIANCE WITH FEDERAL LAW	3
Criterion 2.06: COMPLIANCE WITH STATE LAW	3
Criterion 2.07: COMPLIANCE WITH LOCAL AND MUNICIPAL LAWS, ORDINANCES, AND CODES	4

Criterion 2.01: LEGAL ORGANIZATION AND AUTHORIZATION	
Related Policies:	Distance Education Policy
References:	ACAHM Legal and Regulatory Compliance Checklist ; ACAHM Position Paper: Compliance with Out-of-State Educational Activities

INSTITUTIONAL COMPONENTS

- A. The institution must be a legally organized entity whose primary purpose is to provide graduate education in AHM.
- B. The institution must be authorized to conduct its operations by recognized state approval agencies, and it must comply with applicable state laws in each state in which it operates.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program's parent institution must be authorized to conduct its operations by recognized state approval agencies and applicable state laws in each state in which it operates.
- B. The institution and its ACAHM-accreditable programs must be authorized by all jurisdictions in which it operates to engage in distance education to the extent such authorization is required.

Criterion 2.02: ACCREDITATION STATUS OF PARENT INSTITUTION	
<i>Related Policies:</i>	
<i>References:</i>	ACAHM Legal and Regulatory Compliance Checklist ; ACAHM Position Paper: Multi-Purpose Institution Clarification

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

If the program's parent institution is not institutionally accredited by ACAHM, the parent institution must be accredited or pre-accredited by an agency recognized by the U.S. Secretary of Education.

Criterion 2.03: DEGREE-GRANTING AUTHORITY	
<i>Related Policies:</i>	
<i>References:</i>	ACAHM Legal and Regulatory Compliance Checklist

INSTITUTIONAL COMPONENTS

The institution must comply with recognized state approval agencies and relevant state law with respect to the awarding of degrees, diplomas, and certificates.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program's parent institution must comply with recognized state approval agencies and relevant state law with respect to the awarding of degrees, diplomas, and certificates.

Criterion 2.04: OFF-CAMPUS CONTROL	
<i>Related Policies:</i>	
<i>References:</i>	ACAHM Legal and Regulatory Compliance Checklist ; ACAHM Position Paper: Compliance with Out-of-State Educational Activities ; ACAHM Position Paper: Off-Campus Clinical Training

INSTITUTIONAL COMPONENTS

The institution must assume responsibility for all off-campus educational and co-curricular activities and programs, including activities arranged through agreements with other organizations or individuals.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program's parent institution must assume responsibility for all programmatic, off-campus, educational, and co-curricular activities, including activities arranged through agreements with other organizations or individuals.
- B. For clinical observation and internship training that does not occur on-campus, the location:
 - 1. is subject to a written affiliation agreement outlining roles and responsibilities of all parties and providing for reasonable access to and assessment by program administrators, and
 - 2. requires proof of appropriate insurance.
- C. For clinical externship training that does not occur on-campus, the location:
 - 1. is subject to a written affiliation agreement outlining roles and responsibilities of all parties, and
 - 2. requires proof of appropriate insurance.

Criterion 2.05: COMPLIANCE WITH FEDERAL LAW	
Related Policies:	
References:	ACAHM Legal and Regulatory Compliance Checklist

INSTITUTIONAL COMPONENTS

The institution must continuously comply with all relevant federal laws and regulations.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program must continuously comply with all relevant federal laws and regulations.

Criterion 2.06: COMPLIANCE WITH STATE LAW	
Related Policies:	
References:	ACAHM Legal and Regulatory Compliance Checklist ; ACAHM Position Paper: Compliance with Out-of-State Educational Activities ; ACAHM Position Paper: Off-Campus Clinical Training

INSTITUTIONAL COMPONENTS

The institution must continuously comply with all recognized state approval agencies, and relevant state laws and regulatory requirements, for operation within the state(s) in which it conducts its operations.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program's parent institution must continuously comply with all recognized state approval agencies, and relevant state laws and regulatory requirements, for operation within the state(s) in which it conducts its operations.
- B. For clinical training locations in a state different from where an institution has an ACAHM accredited or pre-accredited main or branch campus (i.e., out-of-state clinical training location), the institution/program must maintain written evidence confirming that the out-of-state clinical training location has received all necessary local and state authorizations/certifications to offer clinical training.

Criterion 2.07: COMPLIANCE WITH LOCAL AND MUNICIPAL LAWS, ORDINANCES, AND CODES	
<i>Related Policies:</i>	
<i>References:</i>	ACAHM Legal and Regulatory Compliance Checklist

INSTITUTIONAL COMPONENTS

The institution must continuously comply with all relevant local and municipal laws, ordinances, codes, and regulatory requirements for operation within the jurisdiction(s) in which it conducts its operations.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program's parent institution must continuously comply with all relevant local and municipal laws, ordinances, codes, and regulatory requirements for operation within the jurisdiction(s) in which it conducts its operations.

ACAHM COMPREHENSIVE STANDARDS AND CRITERIA

STANDARD 3: GOVERNANCE AND ADMINISTRATION

Criterion 3.01: GOVERNANCE STRUCTURE	1
Criterion 3.02: ROLES OF THE GOVERNING BOARD	2
Criterion 3.03: STAKEHOLDER INPUT	3
Criterion 3.04: ORGANIZATION OF ADMINISTRATIVE STAFF	3
Criterion 3.05: ADMINISTRATIVE AND ACADEMIC LEADERSHIP	4

Criterion 3.01: GOVERNANCE STRUCTURE	
Related Policies:	
References:	

INSTITUTIONAL COMPONENTS

- A. The institution must have a clearly delineated governance structure with a governing board that exercises ultimate and general control over the institution's affairs.
- B. The membership of the governing board must effectively represent the needs of the institution's stakeholder groups.
 - 1. Non-profit governing board membership and activities must demonstrate adequate and effective public representation and input.
- C. Governing board members must have primary responsibility to the institution and must not allow personal, political or other influences to interfere with their governing duties.
- D. The governing board of the institution must develop and maintain a set of bylaws, which must include:
 - 1. descriptions of the powers, duties, terms of office, and responsibilities of members of the governing board.
 - 2. a description of meetings, including frequency and format.
 - 3. a clear description of the authority and limitations of the owner(s), the governing board, and the advisory committee, where applicable.
- E. The governing board must develop and follow a code of ethics, including provisions for addressing conflicts of interest and the appearance of such conflicts.
- F. The meetings of the governing board must:
 - 1. be held at regularly stated times.

2. be of sufficient length for the governing board to competently fulfill its responsibilities to the institution and its programs.
3. include prepared agendas.
4. document institutional decision-making in accurate meeting minutes kept on file and readily available at the institution.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program's parent institution must have a governance structure that is appropriate to an institution of post-secondary education offering a graduate level education in AHM.

Criterion 3.02: ROLES OF THE GOVERNING BOARD	
<i>Related Policies:</i>	Glossary
<i>References:</i>	

INSTITUTIONAL COMPONENTS

The governing board of the institution must be responsible for:

- A. governance of the institution.
- B. financial oversight, including but not limited to:
 - a. selecting an independent auditor.
 - b. reviewing and approving the annual budget.
- C. asset management, including protecting assets from fraud and misuse.
- D. fundraising and participating in the development of external relations and resources.
- E. appointing and evaluating the institution's president/CEO and approving President/CEO compensation and benefits.
- F. establishing and maintaining policies, including but not limited to:
 - a. a written whistleblower protection policy.
 - b. a written document retention/destruction policy.
 - c. a written gift acceptance policy.
- G. making strategic decisions and overseeing strategic planning.
- H. approving and ensuring achievement of the institution's mission, strategic goals and objectives.
- J. maintaining corporate minutes.

- K. developing and ensuring processes to review and improve the governing body's own effectiveness.
- L. furthering the non-profit's tax-exempt purpose, *non-profit institutions only*
- M. reviewing the non-profit's IRS Form 990, *non-profit institutions only*

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The governing board of the institution must be responsible for approving significant changes to the program, as defined in the *ACAHM Glossary*.

Criterion 3.03: STAKEHOLDER INPUT	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

Decisions of the governing board must be made based on legal and regulatory requirements and input from the institution's stakeholders and communities of interest.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. Governance-related decisions that affect the program must consider input from relevant program stakeholders.
- B. Consistent with recognized principles of shared governance, the program must demonstrate an effective process for input from students and faculty into institutional decision-making.

Criterion 3.04: ORGANIZATION OF ADMINISTRATIVE STAFF	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

The institution's administrative staff must:

- A. be of a size and structure that is appropriate to the mission and scope of the institution and its programs.

- B. be well-qualified and organized to serve the institution's needs.
- C. have clearly defined roles and responsibilities that support the mission of the institution.
- D. be knowledgeable of their duties and aware of those of the other principal positions in the institution's administrative structure.
- E. be stable regarding employee turnover.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program's administrative staff must:

- A. be of a size and structure that is appropriate to the purpose and scope of the program.
- B. be well-qualified and organized to serve the program's needs.
- C. have clearly defined roles and responsibilities that support the program's purpose, goals, and learning outcomes.
- D. be knowledgeable of their duties and aware of those of the other principal positions in the institution's administrative structure that relate to the program.
- E. be stable regarding employee turnover.

Criterion 3.05: ADMINISTRATIVE AND ACADEMIC LEADERSHIP	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

- A. The institution must employ a president/chief executive officer (CEO) responsible for the entire operation of the institution.
- B. The president/CEO must serve as the liaison between the governing board and institutional staff.
- C. The president/CEO must be appropriately qualified as documented by education, training, and professional experience.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program must have a clearly defined and effective structure for administrative leadership.

- B. The program must have a clearly defined and effective structure for academic leadership.
- C. Individuals serving in leadership roles in the program must be qualified for their positions, as documented by education, training, professional experience and, where relevant, professional licensure.
- D. The program must produce job descriptions that clearly outline expectations and time commitments for all programmatic leadership positions.
- E. The program must employ a qualified and competent individual whose principal employment and responsibilities are to direct the program. The director of the program must be responsible for:
 - 1. providing effective supervision and oversight for the program, including curriculum development and review, faculty recruitment, faculty professional development, and faculty and staff evaluation.
 - 2. the overall assessment of the effectiveness of the program.
- F. The program must employ a qualified and competent individual responsible for direction of the program's clinical education. The director of clinical education for the program must maintain an active current license and/or certification in good standing in the state in which the program operates and be responsible for:
 - 1. providing effective supervision, oversight, coordination, and assessment of all programmatic clinical training activities.
 - 2. the collection, organization, and review of the variety of effectiveness data obtained during clinical training.
 - 3. the overall tracking of students in the clinical training.

ACAHM COMPREHENSIVE STANDARDS AND CRITERIA

STANDARD 4: RECORDS

Criterion 4.01: RECORDKEEPING SYSTEMS.....	1
Criterion 4.02: ACCESS AND STORAGE OF RECORDS	2
Criterion 4.03: PERMANENT ACADEMIC RECORDS	2
Criterion 4.04: CLINICAL RECORDS.....	3
Criterion 4.05: DISCONTINUATION OR CLOSURE	4

Criterion 4.01: RECORDKEEPING SYSTEMS	
Related Policies:	
References:	

INSTITUTIONAL COMPONENTS

- A. The institution must develop and maintain accurate, complete, and secure recordkeeping systems appropriate for institutions offering graduate level healthcare education.
- B. Systems must include student records (i.e., admission applications, enrollment agreements, academic progress and permanent transcripts, financial aid, financial transactions, grievances, etc.), patient health care activities and incident reports, governance-related decision making, faculty evaluation, and personnel/HR.
- C. Systems must collect, compile, and maintain data that will facilitate the improvement of institutional outcomes and the assembly of meaningful records and statistics.
- D. Systems must be regularly monitored and assessed.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program's parent institution must develop and maintain accurate, complete, and secure record keeping systems appropriate for institutions offering graduate level healthcare education.
- B. The program must maintain accurate, complete, and secure programmatic record keeping systems that document student academic progress and that track patient health care activities, including treatment records and incident reports.
- C. Systems must collect, compile, and maintain data that will facilitate the enhancement of program learning outcomes and the achievement of professional competencies.
- D. Systems must be regularly monitored and assessed.

Criterion 4.02: ACCESS AND STORAGE OF RECORDS	
<i>Related Policies:</i>	Access to School Data Policy
<i>References:</i>	

INSTITUTIONAL COMPONENTS

Institutional records in all formats, now known and later developed, shall be securely and accurately maintained and timely recoverable with appropriate access control.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

Program records in all formats, now known and later developed, shall be securely and accurately maintained and timely recoverable with appropriate access control.

Criterion 4.03: PERMANENT ACADEMIC RECORDS	
<i>Related Policies:</i>	
<i>References:</i>	<i>34 CFR Part 99 – Family Educational Rights and Privacy</i>

INSTITUTIONAL COMPONENTS

- A. The institution must maintain accurate and secure permanent academic records.
- B. The institution must safeguard all permanent academic records, and must provide for appropriate backup, storage, and accurate recovery regardless of the record format.
- C. In its recordkeeping activities, the institution must observe right-to-privacy laws and regulations.
 - 1. Non-Title IV financial aid eligible institutions must maintain policies and procedures that protect the privacy rights of students' educational records. At a minimum, these policies must afford students the rights to: access their education records, seek amendments to their educational records and, subject to any legal or regulatory requirements, control to whom their educational records may be disclosed.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. Working with its parent institution, the program must assist in the maintenance of accurate permanent program academic records that document the achievement of program requirements.
- B. Working with its parent institution, the program must assist in safeguarding all permanent academic records that document the achievement of program requirements, and must provide for appropriate backup, storage, and accurate recovery regardless of the record format.

- C. In its programmatic recordkeeping activities, the program must observe right-to-privacy laws and regulations.

Criterion 4.04: CLINICAL RECORDS	
<i>Related Policies:</i>	
<i>References:</i>	<i>Health Insurance Portability and Accountability Act of 1996, Pub. L. No. 104-191</i>

INSTITUTIONAL COMPONENTS

All institutional health records, regardless of format, must be maintained and accessed in accordance with applicable federal and state privacy regulations, and appropriate institutional policies.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. Regardless of format, the program must maintain accurate, secure, and complete clinical records of all patients involved in any part of the educational process.
- B. The program must record and maintain all clinical records in English.
- C. The confidentiality of program clinical records must be maintained according to applicable statutory requirements and relevant professional standards.
 - 1. If an institution is not a HIPAA covered entity, the institution must maintain policies and procedures that protect the privacy of patients' personal health information (medical records). At a minimum, these policies and procedures must permit patients to: examine and obtain copies of their medical records, request corrections to their medical records and, subject to any legal or regulatory requirements, control to whom medical records may be disclosed.
- D. The program must ensure appropriate access to patient records considering relevant legal concerns and the requirements of external regulatory agencies.
- E. The program must implement an appropriate written policy that meets all applicable statutory requirements and relevant professional standards addressing clinical record storage, retention, and recovery.
- F. Charts must meet or exceed all appropriate state regulations regarding chart identification, dating, signatures, and completion timelines.
- G. Charts must be signed by clinical supervisor(s) and clearly identify all students involved in delivering patient care.

Criterion 4.05: DISCONTINUATION OR CLOSURE	
Related Policies:	
References:	

INSTITUTIONAL COMPONENTS

- A. The institution must have a written plan that is compliant with relevant state regulations for the disposition, storage, and accurate recovery of permanent academic records and clinical records, regardless of the records format, if the institution or any of its programs or locations closes.
- B. The records disposition, storage, and data recovery plan must specify the nature, location, and accessibility of all permanent academic records and clinical records.

PROGRAMMATIC COMPONENTS

There are no programmatic components for this criterion.

ACAHM COMPREHENSIVE STANDARDS AND CRITERIA

STANDARD 5: ADMISSIONS AND STUDENT SERVICES

Criterion 5.01: RECRUITMENT AND POLICY PUBLICATION	1
Criterion 5.02: ADMISSIONS	2
Criterion 5.03: TRANSFER CREDITS/TRANSFER STUDENTS	6
Criterion 5.04: NON-MATRICULATED STUDENTS	7
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Criterion 5.07: STUDENT COMPLAINTS AND GRIEVANCES	9

Criterion 5.01: RECRUITMENT AND POLICY PUBLICATION	
Related Policies:	
References:	

INSTITUTIONAL COMPONENTS

- A. The institution must observe honest, ethical, and legal recruiting practices.
- B. The institution must publish its admissions policies, procedures, and practices, including any applicable policies related to:
 - 1. standard admissions,
 - 2. special admissions,
 - 3. advanced placement,
 - 4. credit for prior learning.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program must observe honest, ethical, and legal recruiting practices.
- B. The program must have admissions policies, procedures, and practices that are appropriate for graduate-level professional education and training.
- C. The program must publish its admissions policies, procedures, and practices, including any applicable policies related to:
 - 1. standard admissions,
 - 2. special admissions,
 - 3. advanced placement,
 - 4. credit for prior learning.
- D. The recruitment and admissions policies, procedures, and practices must provide prospective students with the following:

1. a clear description of the program and all elements delivered via distance education, including curriculum, learning outcomes, and appropriate measures of student success (e.g., graduation rates, certification examination passage rates, employment in the field, progression to higher degree).
2. all costs, including tuition, fees (including any additional charges associated with verification of student identity), books, materials, travel for any on-site classroom or clinical requirements, and refund policy.
3. requisite skills for using technology tools (websites, software, and hardware) that are clearly stated and reinforced through admissions, orientation, and matriculation.
4. the processes for authenticating student identification.
5. information regarding what students need to be successful in the program, including pre-requisites, technology, internet connection, and expected amount of engagement per week or per term.
6. expectations for students' engagement as active learners with learning resources, faculty, other students, and assignments as appropriate.
7. support services available to students (including advising, tutoring, mentoring, coaching, and accessibility).
8. information about professional licensure requirements, where appropriate, and advice on licensing in the state(s) of intended employment.
9. information about special opportunities for students.

Criterion 5.02: ADMISSIONS	
<i>Related Policies:</i>	Glossary
<i>References:</i>	20 U.S. Code § 4351.Definitions ; ACAHM Position Paper: English Language Courses and Programs ; ACAHM Position Paper: Recognition of Foreign Qualifications ; ACAHM Position Paper: Entry-Level Professional Doctoral Program Admissions

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program must have admissions policies, procedures, and practices that include explicit English language proficiency requirements, and are consistent with students achieving the statement of purpose and goals of the program.
- B. Admissions policies, procedures, and practices must result in matriculated students who meet the program's admissions criteria and are capable of achieving the professional competencies expected from the program.
- C. The program's admissions policies, procedures, and practices must be regularly and systematically assessed to determine their effectiveness and revised as necessary.

- D. Prospective students must meet the programmatic admissions requirements in effect at the time of their official matriculation.
- E. International applicants whose native language is not English are required to provide proof of English proficiency for admission to degree, certificate, and non-degree graduate programs. This must be demonstrated by one of the following means:

1. The applicant must have completed one of the following:
 - a) four years at a U.S. high school demonstrably delivered in English;
 - b) at least two-years (60 semester credits or 90 quarter credits) of post-secondary education in an institution accredited by an agency recognized by the U.S. Secretary of Education;
 - c) at least two-years (60 semester credits or 90 quarter credits) of post-secondary education demonstrably delivered in English;
 - d) high school or two-years (60 semester credits or 90 quarter credits) of post-secondary education in an institution in one of the following countries or territories: American Samoa; Anguilla; Antigua & Barbuda; Australia; Bahamas; Barbados; Belize; Bermuda; Botswana; British Virgin Islands; Cameroon; Canada (except Quebec); Cayman Islands; Christmas Island; Cook Islands; Dominica; Federated States of Micronesia; Fiji; The Gambia; Ghana; Gibraltar; Grenada; Guam; Guyana; Ireland; Isle of Man; Jamaica; Kenya; Lesotho; Liberia; Malawi; Montserrat; Namibia; New Zealand; Nigeria; Norfolk Island; Papua New Guinea; Philippines; Pitcairn Islands; Sierra Leone; Singapore; South Africa; South Sudan; St. Helena; St. Kitts & Nevis; St. Lucia; St. Vincent & the Grenadines; Swaziland; Tanzania; Trinidad and Tobago; Turks and Caicos Islands; United Kingdom; U.S. Virgin Islands; Uganda; Zambia; Zimbabwe.
 - i. In all cases, English must have been both the language of instruction and the language of the curriculum used.

OR

2. The applicant must have completed one of the following assessments at the required level:

ASSESSMENT TOOL	ACCEPTABLE SCORE
Test of English as a Foreign Language Internet-Based Test (TOEFL® iBT)	Total: 61
International English Language Testing System (IELTS), Academic Format	Overall band: 6
Duolingo English Test	90
China Standard of English Language (CSE)	CSE 6
Cambridge First Certificate in English (FCE)	C
Cambridge English Advanced (CAE)	C
Common European Framework Reference (CEFR)	B2
Occupational English Test (OET)	250, C
Pearson Test of English (PTE), Academic	Overall: 45

MASTER'S-LEVEL AND GRADUATE CERTIFICATE PROGRAMS

- F. The admissions criteria for the program must include the satisfactory completion of at least two (2) years of post-secondary education (defined as 60 semester credits or 90 quarter credits) from an institution accredited or pre-accredited by an agency recognized by the U.S. Secretary of Education. In considering the acceptance of education and training obtained in foreign countries, credits earned at a foreign educational institution must be validated by a recognized educational credentials evaluation service.
- G. A maximum of 30 semester credits (or 50%) of the prerequisite two-year post-secondary education requirement may be earned through prior learning assessment using either, or a combination, of the following assessment techniques: (1) credit by examination using recognized, independently administered, and validated standardized tests and/or (2) assignment of credit for military and corporate training based on recommendations established by the American Council on Education.
- H. Pre-requisite and co-requisite credits must not be counted towards program completion credits.
- J. The admissions criteria for the master's-level **certificate program in Chinese herbal medicine** must include current enrollment in, or the satisfactory completion of, an ACAHM-accredited/pre-accredited entry-level (i.e., master's-level or professional doctoral) program in acupuncture or in acupuncture with a Chinese herbal medicine specialization. In considering the acceptance of education and training obtained in foreign countries, credits earned at a foreign educational institution must be validated by a recognized educational credentials evaluation service.

PROFESSIONAL ENTRY-LEVEL DOCTORAL PROGRAMS

- K. The admissions criteria for the entry-level doctoral program must include the satisfactory completion of at least three (3) years of post-secondary education (defined as 90 semester credits or 135 quarter credits) from an institution accredited or pre-accredited by an agency recognized by the U.S. Secretary of Education. In considering the acceptance of education and training obtained in foreign countries, credits earned at a foreign educational institution must be validated by a recognized educational credentials evaluation service.
 - 1. If the entry-level doctoral program opts to offer a doctoral degree completion track (as defined in ACAHM's Glossary):
 - a) Prospective students must meet all the programmatic standard admissions requirements.
 - b) The admissions criteria for the doctoral degree completion track must also include the satisfactory completion of a master's-level program in acupuncture or acupuncture with a Chinese herbal medicine specialization from an ACAHM accredited/pre-accredited program or institution. In considering the acceptance of education and training obtained in foreign countries, credits earned at a foreign educational institution must be validated to be equivalent to a master's degree by a recognized educational credentials evaluation service.

- c) The program must complete a comprehensive admissions assessment, including transcript analysis, that demonstrates that all applicable master's-level coursework completed is equivalent to published entry-level doctoral program degree requirements in terms of content, rigor, and the attainment of master's-level professional competencies.
 - d) Credits earned more than five (5) years prior to admission may only be accepted for transfer credit by an institution after validating that the student has retained the content knowledge and competencies of the respective course(s) for which transfer credits are being assessed.
 - e) Admission to an entry-level doctoral degree completion track that includes advanced study or specialty concentrations in herbal medicine is further conditioned upon satisfactory completion of ACAHM's masters' core curriculum requirements for herbal training typically evidenced by official transcripts issued by an ACAHM accredited/pre-accredited program or institution.
- L. A maximum of 30 semester credits of the prerequisite three-year post-secondary education requirement may be earned through prior learning assessment using either, or a combination, of the following assessment techniques: (1) credit by examination using recognized, independently administered, and validated standardized tests and/or (2) assignment of credit for military and corporate training based on recommendations established by the American Council on Education.
- M. Prerequisite undergraduate-level education required for admission to the entry-level doctoral program must include chemistry, biology, and psychology. Institutions may offer professional-level programmatic courses in chemistry, biology, and psychology in lieu of program admission prerequisites.
- N. Institutions/programs that confer a master's degree as part of the entry-level doctoral program must ensure that separate enrollment agreements for each degree program are simultaneously executed at the date of initial enrollment; and clearly outline if/when it is possible for a student to be awarded a master's degree without completing the full entry-level doctoral program.

ADVANCED PRACTICE DOCTORAL PROGRAMS

- P. The admissions criteria for the advanced practice doctoral program must include the satisfactory completion of an ACAHM-accredited/pre-accredited entry-level program (i.e., master's or professional doctorate). In considering the acceptance of education and training obtained in foreign countries, credits earned at a foreign educational institution must be validated by a recognized educational credentials evaluation service.
- 1. For advanced practice doctoral programs that include advanced study or specialty concentrations in herbal medicine, the admissions criteria must include the satisfactory completion of the minimum clock hours of instruction stipulated under *criterion 7.02 – Chinese herbal medicine certificate programs*.

2. For advanced practice doctoral programs that do not include advanced study or specialty concentrations in herbal medicine, the admissions criteria must include the satisfactory completion of a minimum of 45 clock hours of instruction (equivalent to three (3) semester credits) of introductory coursework in the fundamentals of Chinese herbal medicine either as a prerequisite or as a co-requisite.
- R. Applications may be accepted for admission into the advanced practice doctoral program from licensed acupuncturists who have not completed an ACAHM-accredited/pre-accredited entry-level program (i.e., master's or entry-level doctoral) provided the program:
1. Demonstrates and documents an acceptable process for validating that the applicant has achieved professional competencies equivalent to an ACAHM-accredited/pre-accredited master's-level program.
 2. Provides opportunities from the institution's master's-level program to rectify any deficiencies identified through the program's entrance evaluation and admission standards. Completion of the identified curriculum can be accomplished through the successful completion of specified courses, or by demonstrating achievement of the specific course objectives through successful completion of challenge examinations.
 3. Does not count prerequisite and co-requisite credits toward degree completion credits.

Criterion 5.03: TRANSFER CREDITS/TRANSFER STUDENTS	
<i>Related Policies:</i>	
<i>References:</i>	ACAHM Position Paper: Recognition of Foreign Qualifications ; ACAHM Position Paper: Entry-Level Professional Doctoral Program Admissions

INSTITUTIONAL COMPONENTS

- A. The institution must publish its policies on awarding of credit by transfer and said policy must comply with all applicable state authorizing and licensing agencies' regulations.
- B. Credits accepted for transfer by the institution must have been earned at a post-secondary institution accredited or pre-accredited by an agency recognized by the U.S. Secretary of Education.
- C. Credits accepted for transfer by the institution must be based on an official transcript received by the institution.
- D. In considering the acceptance of education and training obtained in foreign countries, credits earned at a foreign educational institution must be validated by a recognized education credentials evaluation service.
- E. Transfer students must meet the admissions requirements in effect at the time of their official matriculation.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program's parent institution must publish its policies on awarding of credit by transfer and said policy must comply with all applicable state authorizing and licensing agencies' regulations.
 1. At a minimum, 25 percent of the credits or the recognized clock-hour equivalent required for completion of the program must be earned through coursework offered by and completed at the institution or program granting the credential, with the exception of an approved teach-out plan or agreement.
- B. The program must demonstrate that credits accepted for transfer to meet program graduation requirements are equivalent to published program degree requirements in content, rigor, and credit hour requirements.
- C. The program must demonstrate and document an acceptable process for assuring equivalent competencies for credit accepted via transfer.
- D. Credits accepted for transfer into the program must have been earned at a post-secondary institution accredited or pre-accredited by an agency recognized by the U.S. Secretary of Education.
- E. Credits accepted for transfer into the program must be based on an official transcript received by the institution.
- F. In considering the acceptance of education and training obtained in foreign countries, credits earned at a foreign educational institution must be validated by a recognized educational credentials evaluation service.
- G. Transfer students must meet the program admissions requirements in effect at the time of their official matriculation.
- H. Credits earned more than five (5) years prior to admission may only be accepted for transfer after validating and documenting that the student has retained the content knowledge and competencies of the respective course(s) for which transfer credits are being assessed.

Criterion 5.04: NON-MATRICULATED STUDENTS	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. Non-matriculated students who are enrolled in courses must meet all of the corresponding program's admission requirements and course prerequisites, or they must demonstrate sufficient prior education and experience to successfully complete those courses.
- B. The program must ensure that participation by non-matriculated students in program courses does not adversely affect the quality of instruction.
- C. Credits from ACAHM-accredited/pre-accredited entry-level programs that are earned as a non-matriculated student may not be used to fulfill the corresponding program's admissions requirements stipulated under *criterion 5.02: Admissions*.

Criterion 5.05: STUDENT SERVICES	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

- A. Student services and co-curricular activities must support the fulfillment of the institutional mission.
- B. The institution must ensure that all students have access to well-developed and effective orientation, counseling, academic advising, and career development programs.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The parent institution must ensure that all students have access to well-developed and effective orientation, counseling, academic advising, and career development programs.
 - 1. The program must provide students with appropriate orientation to its distance education offerings, instructions for accessing academic materials, and how to use its learning management platform.
- B. The program must develop and maintain program-specific student services and other student development and co-curricular activities that reflect the nature of the student body being served and support the accomplishment of the program's goals.
- C. Program-specific services and co-curricular activities must be organized to ensure that student needs are met and that the program's learning outcomes are achieved.
- D. The program must ensure that all students enrolled in the program have access to program-specific services.
 - 1. Programs must ensure student accessibility and student privacy for curricular content delivered via distance education.

Criterion 5.06: STUDENT POLICIES	
<i>Related Policies:</i>	
<i>References:</i>	<i>Section 504 of the Rehabilitation Act of 1973; Americans with Disability Act; Title IV of the Civil Rights Act of 1964</i>

INSTITUTIONAL COMPONENTS

- A. The institution must develop and maintain a statement of the rights, privileges, and responsibilities of students.
- B. The institution must develop and maintain policies and procedures relating to student disciplinary proceedings.
- C. The institution's policies and procedures regarding student discipline, discrimination prohibition (e.g., race, disability, religion, sex, etc.), academic progress, grading and refunds must be fair, consistent, published, and made available to students.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program must utilize policies that meet or exceed institutional policies that:

- A. describe the rights, privileges, and responsibilities of program students.
- B. describe program student disciplinary proceedings, discrimination prohibition (e.g., race, disability, religion, sex, etc.), academic progress, grading, and refunds.
- C. are fair, consistent, published, and made available to program students.

Criterion 5.07: STUDENT COMPLAINTS AND GRIEVANCES	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

- A. The institution must maintain fair and efficient policies and procedures for submitting, reviewing and responding to internal student complaints and grievances.
- B. The institution must provide appropriate training to faculty, staff, and students regarding procedures for submitting, reviewing and responding to internal student complaints and grievances.
- C. The institution must maintain a record of all student complaints and grievances during the preceding three-year period.

- D. The institution must be able to demonstrate that any student complaints or grievances were handled in a timely, fair and equitable manner.
- E. The institution must disclose ACAHM's address and telephone number in its published policy on student complaints.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program must consistently apply institutional policies and procedures for submitting, reviewing and responding to internal student complaints and grievances.
- B. The program must provide appropriate training to faculty, staff, and students regarding procedures for submitting, reviewing and responding to internal student complaints and grievances.
- C. The program must maintain a record of all student complaints and grievances during the preceding three-year period.
- D. The program must be able to demonstrate that any student complaints or grievances were handled in a timely, fair and equitable manner.
- E. The program must disclose ACAHM's address and telephone number in its published policy on student complaints.

ACAHM COMPREHENSIVE STANDARDS AND CRITERIA

STANDARD 6: ASSESSMENT AND EVALUATION

Criterion 6.01: SYSTEMATIC EVALUATION	1
Criterion 6.02: ASSESSMENT OF STUDENT LEARNING	2
Criterion 6.03: PROGRAMMATIC REVIEW	2
Criterion 6.04: EVALUATION OF GRADUATE SUCCESS	3
Criterion 6.05: ASSESSMENT METHODS	4
Criterion 6.06: DISSEMINATION OF INFORMATION	4

Criterion 6.01: SYSTEMATIC EVALUATION	
Related Policies:	
References:	

INSTITUTIONAL COMPONENTS

- A. The institution must demonstrate a commitment to ongoing improvement by implementing and documenting systematic institutional evaluation.
- B. The institution must use the results of institutional evaluation activities to determine the extent to which it is fulfilling its mission and achieving its goals (e.g., using regular student assessments to document achievement of all program learning outcomes).

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program must demonstrate a commitment to ongoing improvement by implementing systematic evaluation practices that provide evidence of achieving programmatic goals, learning outcomes, and contributing to the fulfillment of its institution's mission.
- B. The program evaluation and review process must include faculty involvement and evaluation of the effectiveness of teaching and learning.
- C. The results of the evaluation, assessment and review process must be used to improve the program.

Criterion 6.02: ASSESSMENT OF STUDENT LEARNING	
<i>Related Policies:</i>	
<i>References:</i>	Federal Student Aid Handbook

INSTITUTIONAL COMPONENTS

- A. As part of its overall assessment plan, the institution must develop and implement systematic and effective methods designed to assess student learning.
- B. An institution must enforce a satisfactory academic progress (SAP) policy applicable to all students (whether or not they receive federal student aid funds) that meets the U.S. Department of Education's reasonableness test (See *Federal Student Aid Handbook, Volume 1 – Student Eligibility, Chapter 1, school-determined requirements*, specifically referenced and incorporated herein).
- C. Institutions that offer distance education must have processes in place through which the institution establishes that the student who registers in a distance education course or program is the same student who participates in and completes the course or program and receives the academic credit.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program must develop and implement systematic methods designed to assess student learning.
- B. Programs that offer distance education must have processes in place through which the program establishes that the student who registers in a distance education course or program is the same student who participates in and completes the course or program and receives the academic credit.
- C. A variety of measures must be systematically assessed at each stage of the program to ensure that students acquire the competencies outlined in *Criterion 7.04*.
- D. Assessments of student learning must measure the extent to which the students meet course learning outcomes.
- E. Assessments of student learning must be a component used to evaluate whether program learning outcomes are achieved.

Criterion 6.03: PROGRAMMATIC REVIEW	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program must engage in regular, ongoing self-evaluations, with appropriate involvement of all stakeholders.
- B. The program must document its effectiveness in achieving program goals and learning outcomes, the processes leading to those outcomes, and changes resulting from the self-evaluations.
- C. The program must periodically and systematically review its goals and learning outcomes, training model, and curriculum.
- D. Evaluation of program faculty performance must be carried out periodically as part of the program evaluation process.

Criterion 6.04: EVALUATION OF GRADUATE SUCCESS	
<i>Related Policies:</i>	Access to School Data Policy
<i>References:</i>	Program Performance Data Thresholds

INSTITUTIONAL COMPONENTS

- A. The institution must use systematic methods to track and evaluate the career development of its graduates.
- B. Graduate career development data must be used to evaluate the extent to which the institution is fulfilling its mission.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program must use systematic methods to gather and evaluate data related to the professional career development of its graduates, such as job placement rates and earnings.
- B. Graduate career development data must be used to evaluate the extent to which the program is achieving its goals and learning outcomes.
- C. Program performance data must be used to evaluate the extent to which the program is achieving its goals and learning outcomes. Programs shall provide to ACAHM, or ACAHM shall receive performance data as applicable, to include:
 - 1. graduation rate,
 - 2. national certification examination (e.g., NCBAHM modules) pass rates for first-time test takers,
 - 3. state licensure examination pass rates,

4. professional licensure rates of graduates within 12 months of program completion.
- D. If a program falls below published [performance data thresholds](#), ACAHM shall review program performance to determine if the program remains compliant with this criterion.

Criterion 6.05: ASSESSMENT METHODS	
Related Policies:	
References:	

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program must assess students at all approved locations, including all clinical training sites, using equivalent methods.

Criterion 6.06: DISSEMINATION OF INFORMATION	
Related Policies:	
References:	

INSTITUTIONAL COMPONENTS

The institution's evaluation and assessment system must allow for the accurate and timely dissemination of results to stakeholders.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program evaluation and assessment system must allow for the accurate and timely dissemination of information to stakeholders regarding the students' educational progress in achieving:

- A. the programmatic competencies delineated in *Criterion 7.04*, and
- B. the program's learning outcomes.

ACAHM COMPREHENSIVE STANDARDS AND CRITERIA

STANDARD 7: PROGRAM OF STUDY

Criterion 7.01: PROGRAM LEVEL.....	1
Criterion 7.02: MINIMUM PROGRAM LENGTH, CREDITS AND HOURS	2
Criterion 7.03: MAXIMUM PROGRAM LENGTH, CREDITS AND HOURS	6
Criterion 7.04: PROFESSIONAL COMPETENCIES	6
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Criterion 7.01: PROGRAM LEVEL	
Related Policies:	Program Naming Policy
References:	

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program must:

- A. be appropriately named.
- B. be sufficiently rigorous in breadth and depth, and appropriate to graduate professional education in the Acupuncture profession.
- C. employ policies and procedures that ensure reliable and accurate assignment of credit hours and conform to commonly accepted practice in higher education.

- D. ensure that the sequencing, duration, nature, and content of all didactic, practical, and clinical training courses are appropriately integrated and consistent with program purpose, goals, and expected student learning outcomes.
- E. show evidence that it has developed appropriate course prerequisites and that students have completed all prerequisites prior to enrollment in a course.
- F. demonstrate that institutional commitment, the level of instruction, supervision, oversight, and opportunities for graduates, are comparable for:
 - 1. each language track of programs taught in multiple languages.
 - 2. each location for programs taught at multiple locations.
 - 3. all methods of educational delivery (i.e., distance education).

Criterion 7.02: MINIMUM PROGRAM LENGTH, CREDITS AND HOURS	
Related Policies:	Glossary ;
References:	ACAHM Position Paper: ACAHM-Accreditable Doctoral Programs ; ACAHM Position Paper: Entry-Level Professional Doctoral Program Admissions

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

MASTER’S-LEVEL PROGRAMS

These requirements are over and above the 60 semester credits required for admission to the professional master's-level program and must meet ACAHM credit-related definitions.

- A. Professional master’s programs in **acupuncture** must:
 - 1. be at least three (3) academic years in length.
 - 2. provide a minimum of 1,905 clock hours of instruction, including at least:
 - a) 705 clock (contact) hours of instruction in Acupuncture and Herbal Medicine (AHM)¹ theory, diagnosis and treatment techniques in acupuncture, and related studies.

¹ As defined by ACAHM’s *Glossary*, “Acupuncture and Herbal Medicine (AHM)” is a collective term for systems of medicine, incorporating fundamental principles, physiological concepts, theoretical foundations, diagnostic procedures, and treatment modalities, whose fundamental theory originated in China and further developed in a variety of other predominantly Asian cultures. Treatment modalities include acupuncture, herbal medicine, manual therapy, diet/lifestyle counseling, and therapeutic exercise. The Commission uses capitalization of “Acupuncture and Herbal Medicine” to differentiate the system of medicine from the individual treatment modalities.

- b) 660 clock hours of instruction in clinical training, comprised of at least 150 hours in clinical observation and 500 clock hours of instruction in clinical internship.
 - c) 450 clock hours of instruction in biomedical clinical sciences.
 - d) 90 clock hours of instruction in counseling, communication, ethics, and practice management.
- B. Professional master's programs in **acupuncture with a Chinese herbal medicine specialization** must:
- 1. be at least four (4) academic years in length.
 - 2. provide a minimum of 2,625 clock hours of instruction, including at least:
 - a) 705 clock hours of instruction in AHM theory, diagnosis and treatment techniques in acupuncture, and related studies.
 - b) 450 clock hours of instruction in didactic AHM-related herbal studies.
 - c) 870 clock hours of instruction in integrated acupuncture and herbal clinical training, comprised of at least 150 hours in clinical observation and 700 clock hours of instruction in clinical internship.
 - d) 510 clock hours of instruction in biomedical clinical sciences.
 - e) 90 clock hours of instruction in counseling, communication, ethics, and practice management.

PROFESSIONAL ENTRY-LEVEL DOCTORAL PROGRAMS

These requirements are over and above the 90 semester credits required for admission to the professional doctoral program and must meet ACAHM credit-related definitions.

- C. Professional entry-level doctoral programs in **acupuncture** must:
- 1. be at least four (4) academic years in length.
 - 2. provide a minimum of 2,035 clock hours of instruction, including at least:
 - a) 705 clock (contact) hours of instruction in AHM theory, diagnosis and treatment techniques in acupuncture, and related studies.
 - b) 790 clock hours of instruction in clinical training, comprised of at least 150 hours in clinical observation and 500 clock hours of instruction in clinical internship.
 - c) 450 clock hours of instruction in biomedical clinical sciences.
 - d) 90 clock hours of instruction in counseling, communication, ethics, and practice management.
 - e) if the program opts to offer a degree completion track for master's program graduates, the track must be a minimum of 300 clock hours of instruction including a minimum of 130 clock hours of instruction of demonstrated clinical experience that leads to the

development of the programmatic professional competencies outlined in *Criterion 7.04: Professional Competencies*.

3. meet or exceed all existing ACAHM standards and criteria required for master's-level acupuncture programs, in addition to meeting the professional entry-level doctoral standards, criteria, and professional competencies defined in this document.
- D. Professional entry-level doctoral programs in **acupuncture with a Chinese herbal medicine specialization** must:
1. be at least four (4) academic years in length.
 2. provide a minimum of 2,755 clock hours of instruction, including at least:
 - a) 705 clock hours of instruction in AHM theory, diagnosis and treatment techniques in acupuncture, and related studies.
 - b) 450 clock hours of instruction in didactic AHM-related herbal studies.
 - c) 1,000 clock hours of instruction in integrated acupuncture and herbal clinical training, comprised of at least 150 hours in clinical observation and 700 clock hours of instruction in clinical internship.
 - d) 510 clock hours of instruction in biomedical clinical sciences.
 - e) 90 clock hours of instruction in counseling, communication, ethics, and practice management.
 - f) if the program opts to offer a degree completion track for master's program graduates, the track must be a minimum of 300 clock hours of instruction including a minimum of 130 clock hours of instruction of demonstrated clinical experience that leads to the development of the programmatic professional competencies outlined in *Criterion 7.04: Professional Competencies*.
 3. meet or exceed all existing ACAHM standards and criteria required for master's-level acupuncture programs with a Chinese herbal medicine specialization, in addition to meeting the professional entry-level doctoral program standards, criteria, and professional competencies defined in this document.

ADVANCED PRACTICE DOCTORAL PROGRAMS

These hour requirements are over and above the requirements for admission to the advanced practice doctoral program.

E. **Advanced practice doctoral programs** must:

1. be a minimum of 1,200 clock hours of instruction and include a minimum of 200 clock hours of instruction in clinical training, as defined in [ACAHM's Glossary](#).
2. provide opportunities for students to assume in-depth professional responsibilities and demonstrate professional role modeling in supervised activities such as teaching

assignments, participation in administration of services, quality assurance activities, clinical research activities, and clinical supervision responsibilities.

GRADUATE CERTIFICATE PROGRAMS

These requirements are over and above the 60 semester credits required for admission to the professional graduate certificate program and must meet ACAHM credit-related definitions.

F. Professional **Chinese herbal medicine certificate programs** must:

1. be at least two (2) academic years in length
2. provide a minimum of 720 clock hours of instruction, including at least:
 - a) 450 clock hours of instruction in didactic AHM-related herbal studies.
 - b) 210 clock hours of instruction in clinical training, comprised of at least 200 clock hours of instruction in herbal clinical internship training.
 - c) 60 clock hours of instruction in related biomedical clinical sciences.

G. Professional **Asian medical bodywork certificate programs** must:

1. be at least one (1) academic year in length.
2. provide a minimum of 550 clock hours of instruction, including at least:
 - a) 100 clock hours of instruction in AHM theory, diagnosis, and related studies.
 - b) 150 clock hours of instruction in Asian medical bodywork techniques and related treatment modalities.
 - c) 90 clock hours of instruction in clinical training (see *criterion 7.05*).
 - d) 150 clock hours of instruction in biomedical clinical sciences, including musculoskeletal anatomy.
 - e) 60 clock hours of instruction in communication, ethics, and practice management.

H. Professional **medical translation certificate programs** must:

1. be at least two (2) academic years in length.
2. be equivalent to a minimum of 24 semester credits of instruction, including at least:
 - a) 15 semester credits (equivalent to 225 clock hours of instruction) of source language(s) (e.g., Chinese, Korean, Japanese, English) and medical translation.
 - b) 9 Semester credits (equivalent to 135 clock hours) of translation, under faculty mentorship.

Criterion 7.03: MAXIMUM PROGRAM LENGTH, CREDITS AND HOURS	
Related Policies:	
References:	

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program must implement a written policy establishing a maximum time frame in which a student must complete the program, which must be a period that is no longer than 200 percent of the published length of the educational program.

Criterion 7.04: PROFESSIONAL COMPETENCIES ²	
Related Policies:	ACAHM Glossary
References:	CCAHM Clean Needle Technique Manual ; ACAHM Position Paper: ACAHM-Accreditable Doctoral Programs ; Foundational Competencies for Undergraduate Medical Education, AAMC, AACOM, ACGME; 2024

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program must adopt and implement a curriculum that reflects the attainment of the program learning outcomes and the achievement of all relevant required professional competencies, as outlined by this criterion.

MASTER'S-LEVEL PROGRAMS

[Master of Acupuncture (MAc) and Master of Acupuncture with a Chinese herbal medicine specialization (MAcCHM)]

Professionalism: Demonstrate integrity, respect, and ethical reasoning, and promote inclusion of differences in all interactions to improve health care for patients.

² The framework for the professional competencies is adapted from [Foundational Competencies for Undergraduate Medical Education](#). AAMC, AACOM, ACGME; 2024 under the terms of the Creative Commons CC BY-NC license.

The student must demonstrate the ability to:

1. Act with compassion, respect, and integrity in all professional actions to foster trust, prioritize the well-being of patients above self-interest, and support patient-centered care.
2. Safeguard patient privacy, uphold confidentiality, and promote patient autonomy through transparent, informed, shared decision-making.
3. Employ ethical reasoning, including the principles of beneficence, non-maleficence, and justice, to guide professional behavior and clinical decision-making.
4. Take responsibility for errors, act to resolve them constructively, and contribute to a culture of safety, honesty, and continuous improvement.
5. Identify personal, cultural, and systemic biases and actively engage in strategies to mitigate their impact, providing equitable, inclusive, and culturally responsive care for individuals of all backgrounds, lived experiences, and intersectional identities.
6. Demonstrate humility and a willingness to learn from others with different backgrounds and experiences.
7. Complete responsibilities in a thorough, accurate, reliable, and timely manner while upholding legal, regulatory, and ethical standards in all aspects of professional performance.
8. Apply knowledge of professional conduct, protection of patient health information, informed consent, liability, third-party payer requirements, safety regulations, and institutional policies to uphold legal and ethical compliance in all aspects of patient care and practice management.

Patient Care and Procedural Skills: Demonstrate compassionate, effective, holistic, evidence-informed, equitable, and patient-centered care.

The student must demonstrate the ability to:

1. Integrate patient and caregiver context, needs, values, preferences, and experiences into patient care.
2. Perform clinically appropriate patient assessments such as history-taking and physical examination using Acupuncture and Herbal Medicine (AHM)³ methods and biomedical methods.
3. Utilize objective and subjective findings, including functional assessments and reported findings from laboratory or diagnostic imaging tests from healthcare providers, to inform integrative diagnostic reasoning.
4. Identify clinical presentations requiring urgent or emergent referrals. Demonstrate proficiency in basic emergency intervention procedures, including CPR and initial first aid response. Articulate emergency management protocols, documentation, and follow-up procedures appropriate for practice settings.

³ As defined by ACAHM's *Glossary*, "Acupuncture and Herbal Medicine (AHM)" is a collective term for systems of medicine, incorporating fundamental principles, physiological concepts, theoretical foundations, diagnostic procedures, and treatment modalities, whose fundamental theory originated in China and further developed in a variety of other predominantly Asian cultures. Treatment modalities include acupuncture, herbal medicine, manual therapy, diet/lifestyle counseling, and therapeutic exercise. The Commission uses capitalization of "Acupuncture and Herbal Medicine" to differentiate the system of medicine from the individual treatment modalities.

5. Formulate prioritized pattern diagnosis and/or differential diagnosis based on clinical impressions using AHM theory and appropriate biomedical framework.
6. Develop, implement, and modify personalized treatment plans that reflect current clinical evidence, patient goals, and evolving conditions. Plans should integrate appropriate clinical approaches of AHM such as acupuncture, herbal medicine (MAcCHM PROGRAMS), adjunctive therapies, and lifestyle interventions to treat conditions and promote well-being.
7. Maintain professional boundaries and manage inappropriate patient behavior, when necessary.
8. Administer acupuncture and adjunctive techniques (e.g., moxibustion, cupping, gua sha, Asian medical bodywork, dry needling, etc.), employ appropriate patient draping and positioning, and use treatment equipment in a manner that prioritizes patient safety, dignity, hygiene, infection prevention and recognized safety guidelines, including the best practices for acupuncture needle safety and related procedures described in the Clean Needle Technique (CNT) Manual (see CCAHM Clean Needle Technique Manual specifically referenced and incorporated herein) and protocols related to bloodborne pathogens, hazard communication, personal protective equipment, sharps safety, and other healthcare-specific standards.
 - a. ***FOR MAcCHM PROGRAMS:*** Administer herbal medicine-based treatment in a manner that prioritizes patient safety, dignity, appraisal of potential toxicity, side effects, contraindications, and pharmaceutical interactions for herbs and natural products, formulas, and prepared products.
9. Educate patients on behaviors and lifestyle modifications that support health, self-care, and risk reduction. Incorporate community and public health perspectives into wellness strategies.
10. Identify individual, cultural, social, and structural drivers of health that influence patient outcomes. Deliver care that is respectful, trauma-informed, and designed to reduce health disparities.
11. Facilitate timely referrals to other healthcare providers using patient-centered language and appropriate biomedical terminology to support continuity of care.
12. Maintain complete and accurate clinical records for all patients, including treatment notes, referrals, and care summaries. Demonstrate competency in using electronic health records (EHR), practice management software, and digital communication tools to support safe and efficient care.

Medical Knowledge: Apply and integrate foundational knowledge to improve health care for patients.

The student must demonstrate the ability to:

1. Demonstrate knowledge of basic, clinical, pathophysiologic, and social sciences needed for clinical practice.
2. Apply foundational knowledge from AHM and biomedical sciences to clinical scenarios for clinical problem-solving, diagnostic reasoning, treatment planning, and decision-making.
3. Critically appraise information to determine reliability, accuracy, and clinical applicability.
4. Demonstrate knowledge of research design, interpretation, and application to clinical questions.

5. Access knowledge relevant to clinical problems using appropriate resources, including emerging technologies.
6. Describe the roles, skills, and perspectives of other health care disciplines to foster effective collaboration, leading to improved patient care and outcomes.
7. Articulate how psychological, behavioral, social, and structural drivers of health influence disease etiology, progression, and healing. Apply this knowledge to support equitable and context-sensitive clinical reasoning.
8. FOR MACCHM PROGRAMS: The student must demonstrate the ability to:
 - a. describe the fundamental theory underlying the use of herbs, natural products, and formulations.
 - b. accurately articulate properties and functions of herbs and natural products in the materia medica.
 - c. recognize obsolete or restricted herbs and natural products (i.e., endangered species, restricted or toxic substances) and identify appropriate alternatives for said substances.
 - d. accurately articulate properties, functions, principles, dosages, and ingredients of traditional formulations.
 - e. compose and revise formulations of appropriately dosed herbs and natural products based on traditional practice and patient assessment.
 - f. safely and effectively administer herbs and natural products, formulations, and prepared products (i.e., dietary supplements).
 - g. evaluate the efficacy of appropriately administered herbs, natural products, and formulations through the identification and review of current research.
 - h. recall elementary concepts of botany and common chemical constituents of herbs and natural products.
 - i. appraise potential toxicity, side effects, contraindications, and pharmaceutical interactions for herbs and natural products, formulas, and prepared products.
 - j. describe state and federal regulations relevant to the practice of AHM, including scope of practice, vendor compliance with manufacturing standards, and appropriate mechanisms for the reporting of serious adverse events.
 - k. describe the various forms of formulation preparation.
 - l. describe dispensary practices that provide quality assurance, including product storage, facility management, preparation practices, product tracking, and record-keeping.
 - m. recognize ethical issues and evaluate appropriate actions when administering herbs and natural products.

Practice-Based Learning and Improvement Competencies: Integrate feedback, evidence, and reflection to adapt behavior, foster improvement, and cultivate lifelong learning.

The student must demonstrate the ability to:

1. Actively seek, reflect upon, and incorporate formative feedback and assessment data to enhance clinical reasoning, interpersonal effectiveness, and patient care outcomes.
2. Identify personal strengths and limitations through structured reflection, peer input, and performance data to guide learning and professional growth.
3. Set, implement, and periodically reassess measurable learning and improvement goals to support professional development and clinical effectiveness across evolving areas of practice.
4. Locate, critically appraise, and integrate relevant information from classical texts, contemporary texts and research, and clinical databases to improve evidence-informed, patient-centered care plans.
5. Engage in scholarly inquiry and demonstrate a sustained ability to seek, generate, and apply new knowledge that advances clinical practice and the Acupuncture profession.
6. Utilize digital learning tools, electronic health records, research databases, and clinical decision-support technologies to access information, support continuous learning, and improve patient outcomes.
7. Recognize and address personal well-being needs and professional boundaries that may impact professional performance. Demonstrate healthy coping strategies and self-care practices that promote resilience, support work-life integration, and model wellness for peers and patients.

Interpersonal and Communication Skills: Effectively communicate and interact with patients, caregivers, and the health care team to contribute to high-quality, patient-centered care.

The student must demonstrate the ability to:

1. Communicate clearly and accessibly using terminology appropriate for patients, caregivers, colleagues, and health professionals to foster shared understanding and trust.
2. Translate and explain the clinical rationale and expected outcomes of AHM care using patient-centered language and biomedical vocabulary to support interdisciplinary collaboration and informed decision-making.
3. Employ a trauma-informed, patient-centered approach to examination and treatment through active listening and transparent, predictive communication.
4. Communicate clearly, accurately, and compassionately in verbal, nonverbal, written, and electronic formats.
5. Adapt language, tone, and communication strategies to meet the varied needs of patients, caregivers, peers and team members with respect, dignity, and cultural intelligence.
6. Employ effective instructional methods to educate patients, caregivers, peers, and interprofessional team members, supporting shared learning, active participation, and informed engagement in care.

Systems-Based Practice Competencies: Apply knowledge of the larger context of health, including its social and structural determinants, and of systems and resources within and outside of health care, to optimize high-quality, collaborative care for patients.

The student must demonstrate the ability to:

1. Analyze the impact of social, economic, and environmental determinants on patient health and access to AHM-related care and integrate this analysis into clinical decision-making to improve patient outcomes.
2. Identify barriers to patient access to AHM healthcare and strategies for inclusive, culturally responsive, and affordable care for underserved populations.
3. Evaluate the benefits and limitations of current and emerging healthcare technologies (e.g., telehealth, electronic health records, artificial intelligence, machine learning, wearable devices, etc.) and apply this evaluation to enhance the safety, efficiency, and accessibility of AHM services.
4. Describe state and federal laws and regulatory frameworks relevant to AHM, such as scope of practice, safety protocols, and adverse event reporting.
5. Identify and report patient safety concerns, systems issues, and opportunities for quality improvement to promote safe, ethical, and effective AHM care.
6. Interpret healthcare policy, insurance reimbursement, and the financial context of health care.
7. Articulate the strengths and weaknesses of multiple practice and business models, and create:
 - a. practice/office policies and procedures.
 - b. business/professional plans designed to support success in professional practice.
 - c. marketing/outreach plans designed to support success in professional practice.
8. Describe appropriate billing and collection systems for professional practice.
9. Evaluate and synthesize information from varied sources to support collaborative clinical decision making among patients, caregivers, peers, and interprofessional team members.

PROFESSIONAL ENTRY-LEVEL DOCTORAL PROGRAMS

[Doctor of Acupuncture (DAc) and Doctor of Acupuncture with a Chinese herbal medicine specialization (DAcCHM)]

Note that the professional competencies of the entry-level doctoral programs are inclusive of, and build upon, the professional competencies of the master's-level programs. Differentiating factors are identified with underlined text throughout the doctoral competencies.

Professionalism: Demonstrate integrity, respect, and ethical reasoning, and promote inclusion of differences in all interactions to improve health care for patients.

The student must demonstrate the ability to:

1. Act with compassion, respect, and integrity in all professional actions to foster trust, prioritize the well-being of patients and team members above self-interest, and support patient-centered care.

2. Safeguard patient privacy, uphold confidentiality, and promote patient autonomy through transparent, informed, shared decision-making.
3. Employ ethical reasoning, including the principles of beneficence, non-maleficence, and justice, to guide professional behavior and clinical decision-making.
4. Take responsibility for errors, act to resolve them constructively, and contribute to a culture of safety, honesty, and continuous improvement.
5. Identify personal, cultural, and systemic biases and actively engage in strategies to mitigate their impact, providing equitable, inclusive, and culturally responsive care for individuals of all backgrounds, lived experiences, and intersectional identities.
6. Demonstrate humility and a willingness to learn from others with different backgrounds and experiences.
7. Complete responsibilities in a thorough, accurate, reliable, and timely manner while upholding legal, regulatory, and ethical standards in all aspects of professional performance.
8. Apply knowledge of professional conduct, protection of patient health information, informed consent, liability, third-party payer requirements, safety regulations, and institutional policies to uphold legal and ethical compliance in all aspects of patient care and practice management.

Patient Care and Procedural Skills: Demonstrate compassionate, effective, holistic, evidence-informed, equitable, and patient-centered care.

The student must demonstrate the ability to:

1. Integrate patient and caregiver context, needs, values, preferences, and experiences into patient care.
2. Perform clinically appropriate patient assessments such as history-taking and physical examination using AHM methods and biomedical methods.
3. Utilize objective and subjective findings, including functional assessments and reported findings from laboratory or diagnostic imaging tests from healthcare providers, to inform integrative diagnostic reasoning.
4. Explain the clinical indications, risks, and benefits for diagnostic procedures, and evaluate written diagnostic reports, including the range of values that distinguish normal from abnormal findings, as relevant to patient care and communication with other health care providers.
5. Identify clinical presentations requiring urgent or emergent referrals. Demonstrate proficiency in basic emergency intervention procedures, including CPR and initial first aid response. Articulate emergency management protocols, documentation, and follow-up procedures appropriate for practice settings.
6. Formulate prioritized pattern diagnosis and/or differential diagnosis based on clinical impressions using AHM theory and appropriate biomedical frameworks.
7. Develop, implement, and modify personalized treatment plans that reflect current clinical evidence, patient goals, and evolving conditions. Plans should integrate appropriate clinical components of AHM such as acupuncture, herbal medicine (*DACCHM PROGRAMS*), adjunctive therapies, and lifestyle interventions to treat conditions and promote well-being.

8. Maintain professional boundaries and manage inappropriate patient behavior, when necessary.
9. Administer acupuncture and adjunctive techniques (e.g., moxibustion, cupping, gua sha, , Asian medical bodywork, dry needling, etc.), employ appropriate patient draping and positioning, and use treatment equipment in a manner that prioritizes patient safety, dignity, hygiene, infection prevention and recognized safety guidelines, including the best practices for acupuncture needle safety and related procedures described in the Clean Needle Technique (CNT) Manual (see CCAHM Clean Needle Technique Manual specifically referenced and incorporated herein) and protocols related to bloodborne pathogens, hazard communication, personal protective equipment, sharps safety, and other healthcare-specific standards.
 - a. **FOR DACCHM PROGRAMS:** Administer herbal medicine-based treatment in a manner that prioritizes patient safety, dignity, appraisal of potential toxicity, side effects, contraindications, and pharmaceutical interactions for herbs and natural products, formulas, and prepared products.
10. Educate patients on behaviors and lifestyle modifications that support health, self-care, and risk reduction. Incorporate community and public health perspectives into wellness strategies.
11. Identify individual, cultural, social, and structural drivers of health that influence patient outcomes. Deliver care that is respectful, trauma-informed, and designed to reduce health disparities.
12. Facilitate timely referrals, transitions of care, and interprofessional collaboration. Communicate clearly with other healthcare providers using patient-centered language and appropriate biomedical terminology to support collaborative case management and continuity of care.
13. Maintain complete and accurate clinical records for all patients, including treatment notes, referrals, and care summaries. Demonstrate competency in using electronic health records (EHR), practice management software, and digital communication tools to support safe and efficient care.

Medical Knowledge: Apply and integrate foundational knowledge to improve health care for patients.

The student must demonstrate the ability to:

1. Demonstrate knowledge of basic, clinical, pathophysiologic, social, and health systems sciences needed for clinical practice.
2. Apply foundational knowledge from AHM and biomedical sciences to clinical scenarios for clinical problem-solving, diagnostic reasoning, treatment planning, and decision-making.
3. Critically appraise information to determine reliability, accuracy, and clinical applicability.
4. Analyze research design, interpretation, and application to clinical questions. Differentiate between evidence-based medicine and evidence-informed practice and integrate findings into treatment planning.
5. Access knowledge relevant to clinical problems using appropriate resources, including emerging technologies.
6. Describe the roles, skills, and perspectives of other health care disciplines to foster effective collaboration, leading to improved patient care and outcomes.

7. Articulate how psychological, behavioral, social, and structural drivers of health influence disease etiology, progression, and healing. Apply this knowledge to support equitable and context-sensitive clinical reasoning.
8. FOR DAcCHM PROGRAMS: The student must demonstrate the ability to:
 - a. describe the fundamental theory underlying the use of herbs, natural products, and formulations.
 - b. accurately articulate properties and functions of herbs and natural products in the materia medica.
 - c. recognize obsolete or restricted herbs and natural products (i.e., endangered species, restricted or toxic substances) and identify appropriate alternatives for said substances.
 - d. accurately articulate properties, functions, principles, dosages, and ingredients of traditional formulations.
 - e. compose and revise formulations of appropriately dosed herbs and natural products based on traditional practice and patient assessment.
 - f. safely and effectively administer herbs and natural products, formulations, and prepared products (i.e., dietary supplements).
 - g. evaluate the efficacy of appropriately administered herbs, natural products, and formulations through the identification and review of current research.
 - h. recall elementary concepts of botany and common chemical constituents of herbs and natural products.
 - i. appraise potential toxicity, side effects, contraindications, and pharmaceutical interactions for herbs and natural products, formulas, and prepared products.
 - j. describe state and federal regulations relevant to the practice of AHM, including scope of practice, vendor compliance with manufacturing standards, and appropriate mechanisms for the reporting of serious adverse events.
 - k. describe the various forms of formulation preparation.
 - l. describe dispensary practices that provide quality assurance, including product storage, facility management, preparation practices, product tracking, and record-keeping.
 - m. recognize ethical issues and evaluate appropriate actions when administering herbs and natural products.

Practice-Based Learning and Improvement Competencies: Integrate feedback, evidence, and reflection to adapt behavior, foster improvement, and cultivate lifelong learning.

The student must demonstrate the ability to:

1. Actively seek, reflect upon, and incorporate formative feedback and assessment data to enhance clinical reasoning, interpersonal effectiveness, and patient care outcomes.
2. Identify personal strengths and limitations through structured reflection, peer input, and performance data to guide learning and professional growth.

3. Set, implement, and periodically reassess measurable learning and improvement goals to support professional development and clinical effectiveness across evolving areas of practice.
4. Locate, critically appraise, and integrate relevant information from classical texts, contemporary texts and research, and clinical databases to improve evidence-informed, patient-centered care plans.
5. Engage in scholarly inquiry and demonstrate a sustained ability to seek, generate, and apply new knowledge that advances clinical practice and the Acupuncture profession.
6. Utilize digital learning tools, electronic health records, research databases, and clinical decision-support technologies to access information, support continuous learning, and improve patient outcomes.
7. Recognize and address personal well-being needs and professional boundaries that may impact professional performance. Demonstrate healthy coping strategies and self-care practices that promote resilience, support work-life integration, and model wellness for peers and patients.
8. Employ evidence-based medicine and/or evidence-informed practice to improve the patient care process.

Interpersonal and Communication Skills: Effectively communicate and interact with patients, caregivers, and the health care team to contribute to high-quality, patient-centered care.

The student must demonstrate the ability to:

1. Communicate clearly and accessibly using terminology appropriate for patients, caregivers, colleagues, and health professionals to foster shared understanding and trust.
2. Translate and explain the clinical rationale and expected outcomes of AHM care using patient-centered language and biomedical vocabulary to support interdisciplinary collaboration and informed decision-making.
3. Employ a trauma-informed, patient-centered approach to examination and treatment through active listening and transparent, predictive communication.
4. Communicate clearly, accurately, and compassionately in verbal, nonverbal, written, and electronic formats.
5. Adapt language, tone, and communication strategies to meet the varied needs of patients, caregivers, peers and team members with respect, dignity, and cultural intelligence.
6. Employ effective instructional methods to educate patients, caregivers, peers, and interprofessional team members, supporting shared learning, active participation, and informed engagement in care.

Systems-Based Practice Competencies: Apply knowledge of the larger context of health, including its social and structural determinants, and of systems and resources within and outside of health care, to optimize high-quality, collaborative care for patients.

The student must demonstrate the ability to:

1. Analyze the impact of social, economic, and environmental determinants on patient health and access to AHM-related care and integrate this analysis into clinical decision-making to improve patient outcomes.
2. Identify barriers to patient access to AHM healthcare and integrative healthcare and strategies for inclusive, culturally responsive, and affordable care for underserved populations.
3. Evaluate the benefits and limitations of current and emerging healthcare technologies (e.g., telehealth, electronic health records, artificial intelligence, machine learning, wearable devices, etc.) and apply this evaluation to enhance the safety, efficiency, and accessibility of AHM services.
4. Describe state and federal laws and regulatory frameworks relevant to AHM, such as scope of practice, safety protocols, and adverse event reporting.
5. Identify and report patient safety concerns, systems issues, and opportunities for quality improvement to promote safe, ethical, and effective AHM care.
6. Interpret healthcare policy, insurance reimbursement, and the financial context of health care.
7. Articulate the strengths and weaknesses of multiple practice and business models, and create:
 - a. practice/office policies and procedures.
 - b. business/professional plans designed to support success in professional practice.
 - c. marketing/outreach plans designed to support success in professional practice.
8. Describe appropriate billing and collection systems for professional practice.
9. Evaluate and synthesize information from varied sources to support collaborative clinical decision making among patients, caregivers, peers, and interprofessional team members.
10. Describe the role of AHM professionals within current health care systems and the impact of that role on patient care.
11. Interact effectively with other members of the health care team and within that health care system.
12. Describe the prevailing and emerging organization, structure, and responsibilities of the health care team within common medical models.
13. Adapt performance to work effectively within various healthcare systems and interprofessional teams to support collaborative, coordinated, patient-centered care.
14. Facilitate continuity of care through timely referrals and collaborative treatment planning contributing to the development of comprehensive, patient-centered care.
15. Guide a patient into health care systems, e.g., homeless care, elder care, and family services.

ADVANCED PRACTICE DOCTORAL PROGRAM

[Doctor of Acupuncture and Oriental Medicine]

Professionalism: Demonstrate integrity, respect, and ethical reasoning, and promote inclusion of differences in all interactions to improve health care for patients.

The student must demonstrate the ability to:

1. Communicate respectfully and effectively across diverse contexts, adapting to feedback and patient needs while upholding ethical integrity to facilitate effective discussions with team members, patients, and the community.
2. Apply professional leadership practices that support effective communication, collaborative practice, professional advocacy, and teamwork.
3. Articulate the importance of supporting and participating in professional activities and organizations.

Patient Care and Procedural Skills: Demonstrate compassionate, effective, holistic, evidence-informed, equitable, and patient-centered care.

The student must demonstrate the ability to:

1. Employ clinical reasoning, including AHM-based diagnosis development and pattern differentiation, to make therapeutic decisions to manage patients with complex conditions in core and specialty concentration areas.
2. Administer AHM-based treatment, including Chinese herbal medicine as applicable, in core and concentration areas with competence that is qualitatively advanced beyond entry-level.
3. Explain the clinical indications, risks, and benefits for diagnostic procedures, and evaluate written diagnostic reports, including the range of values that distinguish normal from abnormal findings, as relevant to patient care and communication with other health care providers.

Medical Knowledge: Apply and integrate foundational knowledge to improve health care for patients.

The student must demonstrate the ability to:

1. Formulate and justify clinical reasoning utilizing comprehensive, in-depth knowledge of AHM principles, modes of diagnosis, and treatment strategies in the care of patients.
2. Interpret historical cultural perspectives and use them to clarify essential concepts represented in the classical texts of AHM.
3. Demonstrate relevant language terminology skills in the source language (i.e., Mandarin Chinese, classical Chinese, Korean, etc.) sufficient to clarify essential concepts represented in the classical texts of AHM.
4. Describe the roles, skills, and perspectives of other health care disciplines to foster effective collaboration, leading to improved patient care and outcomes.

5. Critically appraise information, including the research base within AHM, to determine reliability, accuracy, and clinical applicability. Describe and apply critical evaluation of common research methodologies within the context of clinical and mechanistic research.
6. Analyze data collection methods, research design, interpretation, and application to clinical questions.

Practice-Based Learning and Improvement Competencies: Integrate feedback, evidence, and reflection to adapt behavior, foster improvement, and cultivate lifelong learning.

The student must demonstrate the ability to:

1. Locate, critically appraise, and integrate relevant information from classical texts, contemporary texts and research, and clinical databases to improve evidence-informed, patient-centered care plans and to develop meaningful contributions to the field.
2. Engage in scholarly inquiry and demonstrate a sustained ability to seek, generate, and apply new knowledge that advances clinical practice and the Acupuncture profession.
3. Effectively use digital learning tools, electronic health records, research databases, and clinical decision-support technologies to access information, support continuous learning, and improve patient outcomes.
4. Propose and complete a viable clinically oriented capstone project including formulating a research question, hypothesis, and design.
5. Effectively disseminate research evidence from the field of AHM to patients, families, community members, the public, and healthcare professionals.

Interpersonal and Communication Skills: Effectively communicate and interact with patients, caregivers, and the health care team to contribute to high-quality, patient-centered care.

The student must demonstrate the ability to:

1. Communicate clearly and accessibly using terminology appropriate for patients, caregivers, colleagues, and health professionals to foster shared understanding and trust.
2. Translate and explain the clinical rationale and expected outcomes of AHM care using patient-centered language and biomedical vocabulary to support interdisciplinary collaboration and informed decision-making.
3. Employ a trauma-informed, patient-centered approach to examination and treatment through active listening and transparent, predictive communication.
4. Communicate clearly, accurately, and compassionately in verbal, nonverbal, written, and electronic formats.
5. Adapt language, tone, and communication strategies to meet the varied needs of patients, caregivers, peers and team members with respect, dignity, and cultural intelligence.
6. Employ effective instructional methods to educate patients, caregivers, peers, and interprofessional team members, supporting shared learning, active participation, and informed engagement in care.

7. Evaluate and synthesize information from varied sources to support collaborative clinical decision making among patients, caregivers, peers, and interprofessional team members.

Systems-Based Practice Competencies: Apply knowledge of the larger context of health, including its social and structural determinants, and of systems and resources within and outside of health care, to optimize high-quality, collaborative care for patients.

The student must demonstrate the ability to:

1. Analyze the impact of social, economic, and environmental determinants on patient health and access to AHM-related care and integrate this analysis into clinical decision-making to improve patient outcomes.
2. Interact effectively with other members of the health care team and within that health care system to forge interdependent relationships with other professions within and outside of the health system to improve care and advance learning.

GRADUATE CERTIFICATE IN CHINESE HERBAL MEDICINE PROGRAM

Note that many of the competencies may be addressed via education completed in entry-level programs prior to or concurrent with enrollment in the Chinese herbal medicine certificate program, and not within the program curriculum itself.

Professionalism: Demonstrate integrity, respect, and ethical reasoning, and promote inclusion of differences in all interactions to improve health care for patients.

The student must demonstrate the ability to:

1. Act with compassion, respect, and integrity in all professional actions to foster trust, prioritize the well-being of patients above self-interest, and support patient-centered care.
2. Safeguard patient privacy, uphold confidentiality, and promote patient autonomy through transparent, informed, shared decision-making.
3. Employ ethical reasoning, including the principles of beneficence, non-maleficence, and justice, to guide professional behavior and clinical decision-making.
4. Take responsibility for errors, act to resolve them constructively, and contribute to a culture of safety, honesty, and continuous improvement.
5. Identify personal, cultural, and systemic biases and actively engage in strategies to mitigate their impact, providing equitable, inclusive, and culturally responsive care for individuals of all backgrounds, lived experiences, and intersectional identities.
6. Demonstrate humility and a willingness to learn from others with different backgrounds and experiences.
7. Complete responsibilities in a thorough, accurate, reliable, and timely manner while upholding legal, regulatory, and ethical standards in all aspects of professional performance.

8. Apply knowledge of professional conduct, protection of patient health information, informed consent, liability, third-party payer requirements, safety regulations, and institutional policies to uphold legal and ethical compliance in all aspects of patient care and practice management.

Patient Care and Procedural Skills: Demonstrate compassionate, effective, holistic, evidence-informed, equitable, and patient-centered care.

The student must demonstrate the ability to:

1. Integrate patient and caregiver context, needs, values, preferences, and experiences into patient care.
2. Perform clinically appropriate patient assessments such as history-taking and physical examination using AHM methods and biomedical methods.
3. Utilize objective and subjective findings, including functional assessments and reported findings from laboratory or diagnostic imaging tests from healthcare providers, to inform integrative diagnostic reasoning.
4. Identify clinical presentations requiring urgent or emergent referrals. Demonstrate proficiency in basic emergency intervention procedures, including CPR and initial first aid response. Articulate emergency management protocols, documentation, and follow-up procedures appropriate for practice settings.
5. Formulate prioritized pattern diagnosis and/or differential diagnosis based on clinical impressions using AHM theory and appropriate biomedical framework.
6. Develop, implement, and modify personalized treatment plans that reflect current clinical evidence, patient goals, and evolving conditions. Plans should integrate appropriate clinical approaches of AHM such as acupuncture, herbal medicine (MAcCHM PROGRAMS), adjunctive therapies, and lifestyle interventions to treat conditions and promote well-being.
7. Maintain professional boundaries and manage inappropriate patient behavior, when necessary.
8. Administer herbal medicine-based treatment in a manner that prioritizes patient safety, dignity, appraisal of potential toxicity, side effects, contraindications, and pharmaceutical interactions for herbs and natural products, formulas, and prepared products.
9. Educate patients on behaviors and lifestyle modifications that support health, self-care, and risk reduction. Incorporate community and public health perspectives into wellness strategies.
10. Identify individual, cultural, social, and structural drivers of health that influence patient outcomes. Deliver care that is respectful, trauma-informed, and designed to reduce health disparities.
11. Facilitate timely referrals to other healthcare providers using patient-centered language and appropriate biomedical terminology to support continuity of care.
12. Maintain complete and accurate clinical records for all patients, including treatment notes, referrals, and care summaries. Demonstrate competency in using electronic health records (EHR), practice management software, and digital communication tools to support safe and efficient care.

Medical Knowledge: Apply and integrate foundational knowledge to improve health care for patients.

The student must demonstrate the ability to:

1. Demonstrate knowledge of basic, clinical, pathophysiologic, and social sciences needed for clinical practice.
2. Apply foundational knowledge from AHM and biomedical sciences to clinical scenarios for clinical problem-solving, diagnostic reasoning, treatment planning, and decision-making.
3. Critically appraise information to determine reliability, accuracy, and clinical applicability.
4. Demonstrate knowledge of research design, interpretation, and application to clinical questions.
5. Access knowledge relevant to clinical problems using appropriate resources, including emerging technologies.
6. Describe the roles, skills, and perspectives of other health care disciplines to foster effective collaboration, leading to improved patient care and outcomes.
7. Articulate how psychological, behavioral, social, and structural drivers of health influence disease etiology, progression, and healing. Apply this knowledge to support equitable and context-sensitive clinical reasoning.
8. Describe the fundamental theory underlying the use of herbs, natural products, and formulations.
9. Accurately articulate properties and functions of herbs and natural products in the materia medica.
10. Recognize obsolete or restricted herbs and natural products (i.e., endangered species, restricted or toxic substances) and identify appropriate alternatives for said substances.
11. Accurately articulate properties, functions, principles, dosages, and ingredients of traditional formulations.
12. Compose and revise formulations of appropriately dosed herbs and natural products based on traditional practice and patient assessment.
13. Safely and effectively administer herbs and natural products, formulations, and prepared products (i.e., dietary supplements).
14. Evaluate the efficacy of appropriately administered herbs, natural products, and formulations through the identification and review of current research.
15. Recall elementary concepts of botany and common chemical constituents of herbs and natural products.
16. Appraise potential toxicity, side effects, contraindications, and pharmaceutical interactions for herbs and natural products, formulas, and prepared products.
17. Describe state and federal regulations relevant to the practice of AHM, including scope of practice, vendor compliance with manufacturing standards, and appropriate mechanisms for the reporting of serious adverse events.
18. Describe the various forms of formulation preparation.
19. Describe dispensary practices that provide quality assurance, including product storage, facility management, preparation practices, product tracking, and record-keeping.

20. Recognize ethical issues and evaluate appropriate actions when administering herbs and natural products.

Practice-Based Learning and Improvement Competencies: Integrate feedback, evidence, and reflection to adapt behavior, foster improvement, and cultivate lifelong learning.

The student must demonstrate the ability to:

1. Actively seek, reflect upon, and incorporate formative feedback and assessment data to enhance clinical reasoning, interpersonal effectiveness, and patient care outcomes.
2. Identify personal strengths and limitations through structured reflection, peer input, and performance data to guide learning and professional growth.
3. Set, implement, and periodically reassess measurable learning and improvement goals to support professional development and clinical effectiveness across evolving areas of practice.
4. Locate, critically appraise, and integrate relevant information from classical texts, contemporary texts and research, and clinical databases to improve evidence-informed, patient-centered care plans.
5. Engage in scholarly inquiry and demonstrate a sustained ability to seek, generate, and apply new knowledge that advances clinical practice and the Acupuncture profession.
6. Utilize digital learning tools, electronic health records, research databases, and clinical decision-support technologies to access information, support continuous learning, and improve patient outcomes.
7. Recognize and address personal well-being needs and professional boundaries that may impact professional performance. Demonstrate healthy coping strategies and self-care practices that promote resilience, support work-life integration, and model wellness for peers and patients.

Interpersonal and Communication Skills: Effectively communicate and interact with patients, caregivers, and the health care team to contribute to high-quality, patient-centered care.

The student must demonstrate the ability to:

1. Communicate clearly and accessibly using terminology appropriate for patients, caregivers, colleagues, and health professionals to foster shared understanding and trust.
2. Translate and explain the clinical rationale and expected outcomes of AHM care using patient-centered language and biomedical vocabulary to support interdisciplinary collaboration and informed decision-making.
3. Employ a trauma-informed, patient-centered approach to examination and treatment through active listening and transparent, predictive communication.
4. Communicate clearly, accurately, and compassionately in verbal, nonverbal, written, and electronic formats.
5. Adapt language, tone, and communication strategies to meet the varied needs of patients, caregivers, peers and team members with respect, dignity, and cultural intelligence.

6. Employ effective instructional methods to educate patients, caregivers, peers, and interprofessional team members, supporting shared learning, active participation, and informed engagement in care.

Systems-Based Practice Competencies: Apply knowledge of the larger context of health, including its social and structural determinants, and of systems and resources within and outside of health care, to optimize high-quality, collaborative care for patients.

The student must demonstrate the ability to:

1. Analyze the impact of social, economic, and environmental determinants on patient health and access to AHM-related care and integrate this analysis into clinical decision-making to improve patient outcomes.
2. Identify barriers to patient access to AHM healthcare and strategies for inclusive, culturally responsive, and affordable care for underserved populations.
3. Evaluate the benefits and limitations of current and emerging healthcare technologies (e.g., telehealth, electronic health records, artificial intelligence, machine learning, wearable devices, etc.) and apply this evaluation to enhance the safety, efficiency, and accessibility of AHM services.
4. Describe state and federal laws and regulatory frameworks relevant to AHM, such as scope of practice, safety protocols, and adverse event reporting.
5. Identify and report patient safety concerns, systems issues, and opportunities for quality improvement to promote safe, ethical, and effective AHM care.
6. Interpret healthcare policy, insurance reimbursement, and the financial context of health care.
7. Articulate the strengths and weaknesses of multiple practice and business models, and create:
 - a. practice/office policies and procedures.
 - b. business/professional plans designed to support success in professional practice.
 - c. marketing/outreach plans designed to support success in professional practice.
8. Describe appropriate billing and collection systems for professional practice.
9. Evaluate and synthesize information from varied sources to support collaborative clinical decision making among patients, caregivers, peers, and interprofessional team members.

GRADUATE CERTIFICATE IN ASIAN MEDICAL BODYWORK PROGRAM

Note that many of the competencies may be addressed via education completed in entry-level programs prior to or concurrent with enrollment in the Asian medical bodywork certificate program, and not within the program curriculum itself.

Professionalism: Demonstrate integrity, respect, and ethical reasoning, and promote inclusion of differences in all interactions to improve health care for patients.

The student must demonstrate the ability to:

1. Act with compassion, respect, and integrity in all professional actions to foster trust, prioritize the well-being of patients above self-interest, and support patient-centered care.
2. Safeguard patient privacy, uphold confidentiality, and promote patient autonomy through transparent, informed, shared decision-making.
3. Employ ethical reasoning, including the principles of beneficence, non-maleficence, and justice, to guide professional behavior and clinical decision-making.
4. Demonstrate awareness of and familiarity with cultural competency, and how cultural issues influence a bodywork session and case management.
5. Complete responsibilities in a thorough, accurate, reliable, and timely manner while upholding legal, regulatory, and ethical standards in all aspects of professional performance.
6. Apply knowledge of professional conduct, protection of patient health information, informed consent, liability, third-party payer requirements, safety regulations, and institutional policies to uphold legal and ethical compliance in all aspects of patient care and practice management.

Patient Care and Procedural Skills: Demonstrate compassionate, effective, holistic, evidence-informed, equitable, and patient-centered care.

The student must demonstrate the ability to:

1. Integrate patient and caregiver context, needs, values, preferences, and experiences into patient care.
2. Perform clinically appropriate patient assessments such as history-taking and physical examination using Asian medical bodywork methods and biomedical methods.
3. Utilize objective and subjective findings, including functional assessments and reported findings from laboratory or diagnostic imaging tests from healthcare providers, to inform integrative diagnostic reasoning.
4. Administer Asian medical bodywork techniques, employ appropriate patient draping and positioning, and use treatment equipment to treat conditions such as acute pain, chronic pain, musculoskeletal conditions, and internal conditions in a manner that prioritizes patient safety, dignity, hygiene, infection prevention and recognized safety guidelines, including the best practices for safety and related procedures described in the Clean Needle Technique (CNT) Manual (see CCAHM Clean Needle Technique Manual specifically referenced and incorporated herein), and protocols related to bloodborne pathogens, hazard communication, personal protective equipment, and other healthcare specific standards.
5. Safely and effectively administer topical/externally applied herbal formulations that are complementary to Asian medical bodywork
6. Identify clinical presentations requiring urgent or emergent referrals. Demonstrate proficiency in basic emergency intervention procedures, including CPR and initial first aid response. Articulate emergency management protocols, documentation, and follow-up procedures appropriate for practice settings.

7. Develop, implement, and modify personalized treatment plans that integrate Asian medical bodywork techniques and principles and reflect current clinical evidence, patient goals, and evolving conditions.
8. Maintain professional boundaries and manage inappropriate patient behavior, when necessary.
9. Describe contraindications for the application of Asian medical bodywork.
10. Educate patients on behaviors and lifestyle modifications that support health, self-care, and risk reduction. Incorporate community and public health perspectives into wellness strategies.
11. Facilitate timely referrals to other healthcare providers using patient-centered language and appropriate biomedical terminology to support continuity of care.
12. Maintain complete and accurate clinical records for all patients, including treatment notes, referrals, and care summaries. Demonstrate competency in using electronic health records (EHR), practice management software, and digital communication tools to support safe and efficient care.

Medical Knowledge: Apply and integrate foundational knowledge to improve health care for patients.

The student must demonstrate the ability to:

1. Explain the fundamental theory underlying the application of Asian medical bodywork.
2. Describe the biomedical pathophysiological process responsible for the patient's clinical presentation.
3. Describe common diagnostic imaging tests such as medical radiology (X-rays), magnetic resonance imaging tests (MRI), computed tomography scans (CT), and ultrasound.
4. Apply foundational knowledge from AHM and biomedical sciences to clinical scenarios for clinical problem-solving, diagnostic reasoning, treatment planning, and decision-making.
5. Access knowledge relevant to clinical problems using appropriate resources, including emerging technologies.

Practice-Based Learning and Improvement Competencies: Integrate feedback, evidence, and reflection to adapt behavior, foster improvement, and cultivate lifelong learning.

The student must demonstrate the ability to:

1. Recognize and address personal well-being needs and professional boundaries that may impact professional performance. Demonstrate healthy coping strategies and self-care practices that promote resilience, support work-life integration, and model wellness for peers and patients.
2. Describe and demonstrate self-cultivation exercises that support the development of hand techniques and body mechanics, including balance, stability, and leverage.
3. Employ a self-cultivation practice, such as Qi Gong, to develop personal structural integrity and apply that understanding to the patient treatment.

Interpersonal and Communication Skills: Effectively communicate and interact with patients, caregivers, and the health care team to contribute to high-quality, patient-centered care.

The student must demonstrate the ability to:

1. Communicate clearly and accessibly using terminology appropriate for patients, caregivers, colleagues, and health professionals to foster shared understanding and trust.
2. Translate and explain the clinical rationale and expected outcomes of Asian medical bodywork care using patient-centered language and biomedical vocabulary to support interdisciplinary collaboration and informed decision-making.
3. Employ a trauma-informed, patient-centered approach to examination and treatment through active listening and transparent, predictive communication.
4. Communicate clearly, accurately, and compassionately in verbal, nonverbal, written, and electronic formats.
5. Adapt language, tone, and communication strategies to meet the varied needs of patients, caregivers, peers and team members with respect, dignity, and cultural intelligence.

Systems-Based Practice Competencies: Apply knowledge of the larger context of health, including its social and structural determinants, and of systems and resources within and outside of health care, to optimize high-quality, collaborative care for patients.

The student must demonstrate the ability to:

1. Describe state and federal laws and regulatory frameworks relevant to Asian medical bodywork, such as scope of practice, safety protocols, and adverse event reporting.
2. Describe appropriate business and practice models, including billing and collection systems, for professional practice.

GRADUATE CERTIFICATE IN MEDICAL TRANSLATION PROGRAM

The student must demonstrate the ability to:

1. access medical information from original sources, including materials in print and digital format.
2. acquire background knowledge on subject matter sufficient to comprehend the content of the source text.
3. appraise equivalence, linguistic, historical, and cultural considerations related to source and target language styles and terms.
4. articulate strengths and weaknesses of various translation strategies and techniques and specific losses that can occur with each.
5. apply medical translation strategies and techniques to accurately convey concepts from source materials in target language.

6. practice ethically and with linguistic integrity, including faithful, accurate and impartial translation and respect for copyrights, intellectual property, and privileged or confidential information.
7. employ digital and traditional resources for source and target language term selection.
8. appraise emerging technology systems for information access and management.
9. translate source language materials into target language that meets academic form and style standards suitable for professional publication.
10. illustrate strategies to secure due recognition for their work and compensation commensurate with their abilities.
11. identify opportunities to enhance capabilities in language, subject field, and professional practice through self-teaching, formal, and informal continuing education.

Criterion 7.05: CLINICAL TRAINING	
Related Policies:	Glossary ;
References:	ACAHM Position Paper: ACAHM-Accreditable Doctoral Programs ; ACAHM Position Paper: Off-Campus Clinical Training

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

MASTER'S-LEVEL, GRADUATE CERTIFICATE, AND PROFESSIONAL ENTRY-LEVEL DOCTORAL PROGRAMS

- A. The clinical internship must be conducted in a clinical internship location (see "Clinical Settings" in [Glossary](#)).
- B. The clinical training must provide sufficient patient contacts to fulfill the professional competencies expected of program graduates.
 1. An **acupuncture program** must ensure that the clinical internship training, conducted under the supervision of program approved supervisors, consists of at least 250 treatments of student-delivered patient care where student interns conduct patient interviews, perform diagnosis and treatment planning, perform appropriate acupuncture and/or related treatments, and follow-up on patients' responses to treatment.
 2. An **acupuncture program with a Chinese herbal medicine specialization** must ensure that the clinical internship training, conducted under the supervision of program approved supervisors, consists of at least 350 treatments of student-delivered patient care where student interns conduct patient interviews, perform diagnosis and treatment planning,

perform appropriate acupuncture and/or herbal medicine treatments, and follow-up on patients' responses to treatment. In at least 100 of the 350 treatments of student-delivered patient care, student interns must conduct patient interviews, perform diagnosis and treatment planning, administer appropriate herbal medicine treatments, and follow-up on patients' responses to treatment.

3. A **Chinese herbal medicine certificate program** must ensure that the clinical internship training, conducted under the supervision of program approved supervisors, consists of at least 100 treatments of student-delivered patient care where student interns conduct patient interviews, perform diagnosis and treatment planning, administer appropriate herbal medicine treatments, and follow-up on patients' responses to treatment.
 4. An **Asian medical bodywork certificate program** must ensure that the clinical training, conducted under the supervision of program approved supervisors, consists of at least 60 treatments of student-delivered patient care, at least 30 minutes in duration, where student interns conduct patient interviews, perform diagnosis and treatment planning, perform appropriate medical bodywork treatments, and follow-up on patients' responses to treatment. The clinical training must be separate and distinct from clinical training in other ACAHM-accredited/pre-accredited programs.
- C. Clinical training must provide a wide range of educational experiences and must place students in internship settings with:
1. an adequate number and variety of supervisors.
 2. an adequate number and variety of patients.
- D. The program must incorporate two or more stages or levels of clinical training, which must be associated with clearly defined outcomes that describe the achievement of competency.

MASTER'S-LEVEL AND PROFESSIONAL ENTRY-LEVEL DOCTORAL PROGRAMS ONLY

- E. The program must ensure that each student fulfills at least 150 hours observing acupuncturists and senior student interns performing AHM therapies in a clinical setting. At least 60 clock hours of clinical observation must include patient diagnosis and treatment performed exclusively by experienced practitioners that have all necessary state authority to perform the AHM therapies and associated faculty duties.

PROFESSIONAL ENTRY-LEVEL DOCTORAL PROGRAMS ONLY

- F. As part of its clinical training, the entry-level doctoral program must provide opportunities for interns to engage in collaborative interactions with other medical providers in appropriate clinical settings.

ADVANCED PRACTICE DOCTORAL PROGRAMS ONLY

- G. The program must offer a concentration in at least one clinical specialty area and/or AHM-related (see [Glossary](#)) modality.
- H. The program must provide in-depth, advanced clinical training in the defined concentration(s) that leads to development of clinical expertise beyond entry-level.

- J. The program must provide in-depth didactic and practical training in the area(s) of concentration sufficient to support the clinical experience.
- K. As part of its clinical training, the program must provide opportunities for interns to engage in collaborative interactions with other medical providers in appropriate clinical settings.
- L. The clinical program must promote the integration of practice and scholarly inquiry.

Criterion 7.06: SYLLABI	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. A syllabus must be prepared for each program course or major unit of instruction including clinical instruction.
- B. Syllabi must be:
 - 1. maintained in the program's curriculum files;
 - 2. distributed to each student in the course/clinical experience; and
 - 3. available to all faculty.
- C. Syllabi must contain all required information needed for a student to successfully complete the requirements of the course, including at minimum:
 - 1. the course description;
 - 2. learning outcomes described in measurable terms;
 - 3. prerequisites for enrolling in the course;
 - 4. an outline of the content of the course and didactic and clinical instruction in enough detail to permit the student to see its full scope;
 - 5. schedule of deadlines for course requirements (e.g., papers, projects, examinations);
 - 6. method(s) of instruction;
 - 7. assessment and grading methods;
 - 8. type of grading system used;
 - 9. attendance policy;
 - 10. procedure for accommodations request;
 - 11. required and recommended reading; and

12. credit hours or clock hours granted, including expected out of class study time and specific out of class requirements.

Criterion 7.07: CONTINUING EDUCATION	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

For institutional offerings of continuing education and/or special instructional activities, provision for such activities must include an adequate administrative structure, a sound financial base, and appropriate facilities.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

Continuing education activities or hours must be non-credit bearing and cannot be converted into academic credits for any purpose.

Criterion 7.08: CAPSTONE PROJECTS	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

There are no institutional components for this criterion.

PROGRAMMATIC COMPONENTS

ADVANCED PRACTICE DOCTORAL PROGRAMS ONLY

A. The doctoral program must require students to demonstrate the achievement of professional competencies under the *Practice-Based Learning and Improvement Domain* as outlined in *criterion 7.04* by completing an acceptable clinically oriented capstone project. The project must demonstrate the necessary knowledge and skills for designing and critiquing approaches to systematic inquiry and the use of qualitative and/or quantitative methods. Capstone projects may include, but are not limited to:

1. Theoretical analyses and review
2. Surveys or analyses of archival data
3. Systematic, qualitative investigations
4. Public policy issues
5. Evaluative research
6. Interpretive translation research

7. Educational research (i.e., professional classroom/clinical pedagogy, patient education)
 8. Case-based research (i.e., Case reports, Case series, Cohort studies, etc.)
 9. Outcomes research/Clinical trials
-
- B. The products from capstone projects must meet academic form and style standards suitable for peer-reviewed professional publications.
 - C. The program must utilize a faculty-based committee to review the capstone projects that includes individuals with demonstrable relevant experience in research, scholarly writing, peer review, publication, and/or clinical practice.
 - D. The program must utilize a comprehensive, committee-based review process for the capstone projects that includes, at a minimum, evaluation of:
 1. the research interest, ethical issues, and methods of addressing such in the research,
 2. data gathering methods,
 3. progress toward completion, and
 4. final project content, format, and delivery.

ACAHM COMPREHENSIVE STANDARDS AND CRITERIA

STANDARD 8: FACULTY AND GUEST LECTURERS

Criterion 8.01: CORE FACULTY	1
Criterion 8.02: CREDENTIALS	2
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Criterion 8.01: CORE FACULTY	
Related Policies:	
References:	ACAHM Position Paper: Faculty and Guest Lecturer Clarification

INSTITUTIONAL COMPONENTS

The institution must have written position descriptions for its faculty, stating at a minimum the responsibilities of the position and the required qualifications.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program must employ an identifiable core group of qualified full- and/or part-time didactic and clinical faculty members who have regular and ongoing responsibility for the design, delivery, and evaluation of the program, and are involved in program development, review, and governance.
- B. The program's core faculty must:
 1. be sufficient in numbers for their academic and professional responsibilities to the program;
 2. function as an integral part of the program, including engaging in curriculum planning and development, and program evaluation;
 3. have theoretical perspectives, as well as academic and applied experience, appropriate to the program's goals and expected student learning outcomes;
 4. demonstrate substantial competence and have recognized credentials and experience in those areas which are at the core of program goals and expected student learning outcomes;
 5. be available to function as appropriate role models for students in their learning, socialization into the discipline and profession, acquisition of knowledge, and competencies consistent with the program's training goals; and
 6. be accessible to students to provide them with a level of guidance and supervision that actively encourages timely completion of the program.

Criterion 8.02: CREDENTIALS	
<i>Related Policies:</i>	Glossary
<i>References:</i>	ACAHM Position Paper: Faculty and Guest Lecturer Clarification

INSTITUTIONAL COMPONENTS

The institution's faculty and guest lecturers must possess recognized credentials and demonstrate competence in those areas they are assigned to teach.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

A. Program faculty and guest lecturers must:

1. possess appropriate recognized credentials,
2. have relevant professional experience,
3. demonstrate substantial competence in the areas they are assigned to teach,
4. maintain an active current license and/or certification in good standing in the state in which the program operates, if involved with clinical training and/or patient care

MASTER'S-LEVEL AND GRADUATE CERTIFICATE PROGRAMS

B. For each faculty member who does not possess at least a master's level degree, certificate or diploma, the program must document the method(s) by which it determined the instructor's competence, including relevant education and training and significant experience and leadership in their areas of instruction, to teach at the master's level.

PROFESSIONAL ENTRY-LEVEL DOCTORAL AND ADVANCED PRACTICE DOCTORAL PROGRAMS

C. The doctoral program must employ didactic and clinical faculty who are credentialed at the doctoral level. Within seven (7) years from the date of approval to begin a doctoral program, the program or institution must demonstrate that a majority of faculty teaching in the program have achieved doctoral-level degrees or the terminal degree in the field in which they are currently teaching.

Criterion 8.03: POLICIES AND PROCEDURES	
<i>Related Policies:</i>	
<i>References:</i>	Fair Labor Standards Act ; Americans with Disabilities Act ; ACAHM Position Paper: Faculty and Guest Lecturer Clarification

INSTITUTIONAL COMPONENTS

- A. The recruitment, hiring, appointment, promotion, and retention of faculty members and guest lecturers must be described in institutional policy documents, job descriptions, and procedures.
- B. Policies and procedures relating to the recruitment, hiring, appointment, promotion and retention of faculty members and guest lecturers must address pertinent legal requirements in areas of Fair Labor Standards Act, non-discrimination, equal employment opportunity, disability accommodation, and other applicable federal, state, and local employment laws. Nothing in this Standard or Criterion requires preferential hiring, quotas, or identity-based employment decisions.
- C. Each faculty member and guest lecturer must have a letter of appointment and/or contract that specifies the terms and duties of employment including expected time commitment for each identified duty.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. Policies and procedures pertaining to faculty and guest lecturer work must be described in published documents that are made available to program faculty and guest lecturers.
- B. Program policies pertaining to teaching loads must be reasonable and justified by factors such as the type and method of instruction; the size of classes; the level of instruction; the qualifications of the instructor; the academic advising, committee membership, and guidance and student organizations assigned; and the other administrative, research, publication, and community relations responsibilities of the instructor.
- C. Programs must utilize a systematic faculty evaluation process.

Criterion 8.04: CONDITIONS OF SERVICE	
<i>Related Policies:</i>	
<i>References:</i>	ACAHM Position Paper: Faculty and Guest Lecturer Clarification

INSTITUTIONAL COMPONENTS

- A. Terms and conditions of service for faculty at the institution must be adequate, equitable, and administered ethically.
- B. The institution's faculty members must be accorded academic freedom in their work.
- C. Meaningful opportunities for professional growth and development must be made available to the institution's faculty.

- D. Institutional policies and employment contracts must address preparation time if faculty members are involved in instructional work.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. Terms and conditions of service for program faculty must be adequate, equitable, and administered ethically.
- B. The program's faculty members must be accorded academic freedom in their work.
- C. Meaningful opportunities for professional growth and development must be made available to the program's faculty.
- D. Program policies and/or employment contracts must address preparation time if faculty members are involved in instructional work.

Criterion 8.05: FACULTY COMMUNICATION	
Related Policies:	
References:	ACAHM Position Paper: Faculty and Guest Lecturer Clarification

INSTITUTIONAL COMPONENTS

- A. Provision must be made for regular and open communication among members of the faculty, and between the faculty and administrative officers of the institution.
- B. The institution's faculty must meet at minimum once per academic term to consider curriculum and educational policies and issues.
- C. Complete and accurate minutes of institutional faculty meetings must be maintained and kept in permanent files at the institution.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. Provision must be made for regular and open communication among members of the program faculty, and between program faculty and members of the program administration.
- B. Program faculty must meet at minimum once per academic term to consider curriculum and educational policies and issues.
- C. Program faculty must have appropriate input into the curriculum development process.
- D. Complete and accurate minutes and attendance of program faculty meetings must be maintained and kept in permanent files.

ACAHM COMPREHENSIVE STANDARDS AND CRITERIA

STANDARD 9: INSTITUTIONAL AND PROGRAM RESOURCES

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Criterion 9.02: GENERAL SUFFICIENCY AND STABILITY OF RESOURCES.....	2
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Criterion 9.01: FINANCIAL MANAGEMENT	
Related Policies:	
References:	

INSTITUTIONAL COMPONENTS

- A. The institutional financial management system must support full audits and review by an independent certified public accountant.
- B. Each year, at minimum, a reviewed institutional financial statement must be prepared that describes the current state of institutional finances.
- C. The institution must utilize an accrual basis of accounting.
- D. Institutions organized as sole proprietorships must have and maintain financial records, management systems, and bank accounts separate and distinct from any other enterprise owned by the proprietor.

PROGRAMMATIC COMPONENTS

There are no programmatic components for this criterion.

Criterion 9.02: GENERAL SUFFICIENCY AND STABILITY OF RESOURCES	
<i>Related Policies:</i>	
<i>References:</i>	Federal Student Aid Handbook

INSTITUTIONAL COMPONENTS

The institution must be financially stable with resources sufficient to: ensure long-term viability; support its mission; fund its programs, commitments and priorities; and, respond to financial emergencies as evidenced by:

- A. a full independent institutional audit with management letter by a licensed CPA for the most recent year. The audit must provide a detailed, accurate picture of the institution's financial status, and must include a classified balance sheet statement, the statement of revenue and expenditures, changes in fund balance and/or financial position, and a Financial Responsibility Composite Score (see *criterion 9.03*). Audit must confirm financial viability with evidence of follow-up on any concerns cited in management letter;
- B. at minimum, a financial review for all other years during the last accreditation cycle;
- C. a reviewed and approved annual budget which must include assumptions and three-year projections;
- D. adequate financial resources to meet debt-service requirements of short-term and long-term indebtedness without adversely impacting the institution; and
- E. a detailed explanation and a realistic plan for eliminating any accumulated deficit.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The program's resources must be aligned with, and sufficient to support, its educational purpose, goals, and learning outcomes, as evidenced by:

- A. an approved, annual program budget that includes a realistic projection of program revenue and expenditures;
- B. documentation of revenues and expenditures for the immediate prior and current years; and
- C. projected revenues and expenditures for the next fiscal year.

Criterion 9.03: FINANCIAL RESPONSIBILITY	
<i>Related Policies:</i>	
<i>References:</i>	Federal Student Aid Handbook

INSTITUTIONAL COMPONENTS

The institution's financial systems, ratios, outcomes, indicators, policies, and procedures must maintain standards of financial responsibility as demonstrated by the Financial Responsibility Composite Score utilized by the US Department of Education (See *Federal Student Aid Handbook, Volume 2 – School Eligibility and Operations, Chapter 11 – Financial Standards*, specifically referenced and incorporated herein) or other ACAHM-approved metric.

PROGRAMMATIC COMPONENTS

There are no programmatic components for this criterion.

Criterion 9.04: FINANCIAL PLANNING AND CONTROL	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

- A. The institution must demonstrate resource planning and development that includes realistic budgeting, enrollment management, and diversification of revenue sources. Resource planning must be integrated with all other institutional planning.
- B. The process by which the institution's annual budget is established, and resources allocated, must be clearly defined and consistently implemented.
- C. The budget must be reviewed and approved by the institution's governing body.
- D. The institution's annual budget must provide a realistic projection of the institution's annual revenue and expenditures.
- E. The institution must project its anticipated expenditures and revenues for at least a three (3) year period.
- F. The institution's budget must include notes, when necessary, that explain the assumptions on which the projected figures are based.
- G. The institution must have control of its financial resources and budgetary process and be free of undue influence or pressure from owners, shareholders, external funding sources or agencies.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The program budget must be developed based on resource planning and development that includes realistic budgeting, enrollment management, and diversification of revenue sources.
- B. The process by which the program's annual budget is established and approved, and resources allocated, must be clearly defined and consistently implemented.
- C. Program leadership must have input into the annual programmatic budgeting process.
- D. Program leadership must have sufficient control of the program budget.

Criterion 9.05: FINANCIAL AID OPERATION	
<i>Related Policies:</i>	
<i>References:</i>	Federal Student Aid Handbook ; Legal and Regulatory Checklist

INSTITUTIONAL COMPONENTS

The financial aid operation must meet all federal aid requirements and be capably administered, as documented by the appropriate Department's compliance audit, financial statement audit, and/or the Office of the Inspector General (OIG) when there is concern over an institution's administration of federal student aid programs.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The parent institution's financial aid operation must meet all federal aid requirements and be capably administered, as documented by the appropriate Department's compliance audit, financial statement audit, and/or the Office of the Inspector General (OIG) when there is concern over an institution's administration of federal student aid programs.

Criterion 9.06: COHORT DEFAULT RATE	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

Institutions must closely monitor student borrowing and implement effective default prevention programs as soon as possible so as not to face Department sanctions for high cohort default rates and lack of administrative capability.

PROGRAMMATIC COMPONENTS

There are no programmatic components for this criterion.

Criterion 9.07: REFUND POLICY	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

Institutions must clearly define, publish, and follow a fair and equitable refund policy for unearned tuition that complies with applicable state and federal laws and regulations.

PROGRAMMATIC COMPONENTS

There are no programmatic components for this criterion.

Criterion 9.08: CAMPUS AND PROGRAM FACILITIES	
<i>Related Policies:</i>	
<i>References:</i>	Americans with Disabilities Act

INSTITUTIONAL COMPONENTS

- A. Campus facilities must be appropriately accessible and sufficient to meet the institutional mission, and to support its academic programs, faculty, students, internal and external commitments, and priorities.
- B. General provisions must be made for the cleaning, repair, and maintenance of the institution's buildings and grounds.
- C. Responsibilities for care of grounds, security, fire protection, utilities, and facilities upkeep must be appropriately assigned.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. Program facilities must be appropriately accessible and sufficient to meet the program's statement of purpose, and to support its faculty, students, goals and learning outcomes.
 - 1. Clinical facilities must have adequate space and equipment to provide for safe patient care, appropriate maintenance of equipment, storage of supplies and records, intern work areas, and waste management.
- B. The program must ensure provision for cleaning, repair, safety, and maintenance of facilities used by the program.

Criterion 9.09: LEARNING RESOURCES SYSTEM (LIBRARY)	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

- A. An institution's learning resource system must be sufficient to meet the institution's mission and support its instructional programs and degree level(s) offered.
- B. The learning resource system must include appropriate materials in sufficient quantity and scope to meet the purpose, goals, and expected student learning outcomes of each program, with particular emphasis on information literacy, and include such elements as: relevant and current texts and periodicals; research journals and databases; standard works of reference; multi-media and/or electronic resources; electronic library resource technologies; and other resource materials necessary to adequately serve the student body.
- C. The learning resource system must be managed by dedicated, qualified personnel with sufficient experience to provide oversight and sufficient availability to support faculty and student access to learning resources.
- D. An institution must have written policies and procedures for the ongoing development of its learning resource system as part of its institutional improvement program.
- E. An institution must demonstrate sufficient support and budgetary allocations for the learning resource system.
- F. The learning resource system must have adequate space and access time, appropriate to the number of potential users.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

- A. The parent institution's learning resource system must be sufficient to support the program's purpose, goals, and learning outcomes for the degree level(s) offered.
- B. The learning resource system must include appropriate materials in sufficient quantity and scope to meet the purpose, goals, and expected student learning outcomes of the program, with particular emphasis on information literacy, and include such elements as: relevant and current texts and periodicals; research journals and databases; standard works of reference; multi-media and/or electronic resources; electronic library resource technologies; and other resource materials necessary to adequately serve the student body.
- C. The learning resource system must be managed by dedicated, qualified personnel with sufficient experience to provide oversight and sufficient availability to support faculty and student access to learning resources.
- D. The parent institution must demonstrate sufficient support and budgetary allocations for program-specific resources within the learning resource system.

- E. The learning resource system must have adequate space and access time, appropriate to the number of potential users.

Criterion 9.10: TECHNOLOGICAL RESOURCES	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

Information technology resources must be sufficient to meet the institution's mission, to support its academic programs, commitments, and priorities.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

Information technology resources must be sufficient to support program delivery, program goals, and expected student learning outcomes.

ACAHM COMPREHENSIVE STANDARDS AND CRITERIA

STANDARD 10: PUBLICATIONS AND ADVERTISING

Criterion 10.01: ACCURACY, HONESTY AND CLARITY OF INFORMATION	1
Criterion 10.02: CATALOG	2

Criterion 10.01: ACCURACY, HONESTY AND CLARITY OF INFORMATION	
Related Policies:	Public Disclosure Policy
References:	ACAHM Legal and Regulatory Compliance Checklist

INSTITUTIONAL COMPONENTS

All institutional published materials and electronic media must:

- A. be honest, complete, accurate, and clear.
- B. be readily available to relevant stakeholders.
- C. accurately identify current programs and services where applicable.
- D. accurately list the professional education and credentials of current faculty, guest lecturers, and leadership where applicable.
- E. clearly and accurately differentiate faculty and guest lecturers.
- F. accurately describe employment, career, and licensure opportunities for graduates where applicable.
- G. report the institutional accreditation status and relationship with the Commission using only the language provided by the Commission where applicable.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

All programmatic published materials and electronic media must:

- A. be honest, complete, accurate, and clear.
- B. be readily available to relevant stakeholders.
- C. accurately identify current programs, services, courses, and faculty where applicable.
- D. accurately list the professional education and credentials of current faculty, guest lecturers, and leadership where applicable.

- E. clearly and accurately differentiate faculty and guest lecturers.
- F. accurately describe employment, career, and licensure opportunities for graduates where applicable.
- G. accurately describe the program's use of, and requirements for, distance education delivery of course content.
- H. report the institutional accreditation status and relationship with the Commission using only the language provided by the Commission where applicable.

Criterion 10.02: CATALOG	
<i>Related Policies:</i>	
<i>References:</i>	

INSTITUTIONAL COMPONENTS

- A. The institution must publish a catalog or comparable official publication(s).
- B. The institution's catalog or comparable official publication(s) must be made available to current students, prospective students, and the public.
- C. The institution's official publication(s) must honestly and accurately describe the institution's programs.
- D. The institution's official publication(s) must include, but is not limited to, current:
 - 1. mission statement;
 - 2. description of all programmatic offerings;
 - 3. governance structure, including membership of the governing board and, if appropriate, the advisory committee;
 - 4. administrative leadership;
 - 5. opportunities and requirements for financial aid, if applicable;
 - 6. procedures for discipline and/or dismissal for academic or other reasons;
 - 7. grievance procedures for students;
 - 8. non-discrimination policy;
 - 9. available learning and other physical resources.

PROGRAMMATIC COMPONENTS

ALL PROGRAMS

The institutional catalog or official publication(s) that refer to the program must include the program's:

1. statement of purpose and goals;
2. learning outcomes;
3. admissions requirements and procedures;
4. admissions and transfer credit policies;
5. rules and regulations for student conduct;
6. attendance policy;
7. grading policy;
8. satisfactory performance requirements;
9. completion requirements;
10. tuition and fee structures;
11. equitable refund policies;
12. administrative staff and academic leadership;
13. academic calendar;
14. curriculum, including credits or clock hours and descriptions of each course;
15. schedule of course offerings by academic term;
16. professional education and credentials of program faculty, guest lecturers, and leadership;
 - a) for listed doctoral degrees, designation of the country of origin, other than the U.S., in which the degree is conferred must be listed, e.g., Ph.D. (China), M.D. (China);
 - b) publications and advertising must clearly and accurately differentiate faculty and guest lecturers.
17. information regarding whether completion of the program would be sufficient to meet the requirements for professional licensure or certification, including:
 - a) a list of all states for which the institution has determined that its curriculum meets the state educational requirements for licensure or certification; and
 - b) a list of all states for which the institution has determined that its curriculum does not meet the state educational requirements for licensure or certification.

ACAHM Comprehensive Standards – REVISION HISTORY

Date (YYMMDD)	Criterion	Nature of Revision	Responsible Official/ Approved By
260403	8.03	Removed reference to “affirmative action employment practices”	Executive Director
260228	10.02	Removed option for list of states where licensure eligibility was not determined	Commission
260228	10.01	Removed requirement to differentiate “core” faculty; added requirement for distance education disclosure	Commission
260228	9.08	Added “appropriately accessible” requirement	Commission
260228	8.04	Modified prep time requirement to be in policy or contract	Commission
260228	8.03	Added requirement for disability accommodation	Commission
260228	8.02	Added requirement for clinical faculty licensure	Commission
260228	7.05	Added herbal medicine patient contact requirement for MACCHM and DAcCHM programs; added requirement for adequate number and variety of patients	Commission
260228	7.04	Complete revision of all programs’ professional competencies; added competencies for certificate in medical translation	Commission
260228	7.02	Modified requirements for each program by removing credit hour equivalents (clock hours of instruction unchanged); relocated formerly separate doctoral completion requirements under entry-level doctoral program requirements; added two academic year length requirement for CCHM programs; added requirements for certificate in medical translation	Commission
260228	7.01	Added requirement for appropriate program name	Commission
260228	6.04	Updated NCBAHM name; added professional licensure requirement	Commission
260228	5.08	Criterion deleted; requirements relocated to 5.02	Commission
260228	5.07	Minor clarification edits	Commission
260228	5.06	Added “discrimination prohibition”	Commission
260228	5.05	Added requirements for distance education orientation and accessibility	Commission
260228	5.04	Clarified requirements re: credits as non-matriculated students apply to all programs	Commission
260228	5.03	Changed transfer credit limitations	Commission
260228	5.02	Modified admission requirements from “undergrad to grad” to “post-secondary”; relocated entry-level doctoral program completion track requirements from former criterion 5.08	Commission
260228	5.01	Added requirements related to distance education disclosures	Commission
260228	4.04	Changed charting requirements to identify students	Commission
260228	3.02	Modified board responsibilities to differentiate non-profit requirements	Commission
260228	3.01	Changed governing “body” to governing “board”; clarified requirement for public representation; modified requirement for code of ethics	Commission
260228	2.04, 2.06	Added requirements for off-campus clinical training	Commission
260228	1.03, 1.04	Clarified that each program must have goals and learning outcomes	Commission
260228	1.02	Clarified that each program must have a statement of purpose; removed requirement to reference training health care providers	Commission
260228	Throughout	“East Asian Medicine/EAM” changed to “Acupuncture and Herbal Medicine/AHM”; relevant Position Papers linked as References	Commission
240220	1.03	Revised and expanded goals for adv. pract. doctoral programs	Commission
240220	7.02	Changed adv. pract. doctoral program requirement from 650 hours of clinical “experience” to 200 hours of clinical “training”; item F.2 moved from 7.05 to 7.02	Commission

240220	7.04	Clarified competencies are designated by program not by level of instruction; full revision of adv. pract. doctoral competencies	Commission
240220	7.05	Adv. pract. doct. program requirement for professional role-modeling deleted and relocated to 7.02.F.2	Commission
240220	7.08	Changed criterion name from “Clinical Research Projects” to “Capstone Projects”; revised capstone requirements	Commission
240220	8.02	Changed doctoral program requirement from 10 years to 7 years post approval for majority of faculty (change from “core faculty”)	Commission
240220	Throughout	Removed DAOM and DAc acronyms for all references to doctoral programs	Executive Director
220805	3.01	Changed “advisory board” to “advisory committee”	Executive Director
220805	Throughout	Added “entry-level” to references to professional doctoral programs	Executive Director
220722	7.02	Added certificate in Asian medical bodywork program requirements	Commission
220722	7.04	Added certificate in Asian medical bodywork program professional competencies; separated advanced practice doctoral program competencies from entry-level program competencies	Commission
220722	7.05	Added certificate in Asian medical bodywork program requirements	Commission
220121	5.02	Added English language proficiency requirements for international applicants for all programs. Removed program-specific language proficiency requirements. Added subcomponent regarding allowance of credits earned through prior learning assessment under DAc program admission requirements.	Commission
211105	3.05	Removed “ <i>whose principal employment</i> ” from Institutional Component regarding president/CEO	Commission
211105	5.04	Added “ <i>Credits ... that are earned as a non-matriculated student may not be used to fulfill the corresponding program’s admissions requirements...</i> ”	Commission
211105	Throughout	Replaced “ACAOM” with “ACAHM.” Replaced terms “Oriental” and “AOM” with “East Asian medicine” and “EAM” where appropriate.	Commission
200917	7.02	minor, nonsubstantive text edits	Executive Director
200917	10.02	Updated disclosure requirements for programs leading to professional licensure or certification	Executive Director
200527	Throughout	Replaced terms “Oriental” and “AOM” with “acupuncture,” “Chinese herbal medicine,” “ACAOM-accreditable,” “with a specialization in Chinese herbal medicine” where appropriate; clarified that “Master’s Level” programmatic components are relevant to certificates; replaced “post-professional” with “advanced practice” for DAOM; replaced “PD” abbreviation with “DAc”	Executive Director
200527	Introduction	Removed history of standard adoptions (directed to Revision History document); added certificate to master’s-level ACAOM-Accreditable Programs section; added that all distance education must comply with standards	Commission
200527	2.03	Added “certificates”	Executive Director
200527	3.04	Added “ <i>be stable regarding employee turnover</i> ”	Commission
200527	4.04	Added “ <i>Charts must be signed by clinical supervisor(s) and student intern(s).</i> ”	Commission
200527	5.02	Relocated certificate requirements to master’s-level; removed outdated English proficiency requirements; revised master’s-level program English proficiency requirements	Commission

200527	5.08	Changed "transcript analysis" to "admissions assessment," changed "credit hour requirements" to "the attainment of master's-level professional competencies"; changed criterion title from "doctorate" to "doctoral"	Commission
200527	6.02	Added institution/program requirement for distance education to validate student identity	Commission
200527	6.05	Removed " <i>Methods of assessment must document accomplishment of specified program competencies and course and program learning outcomes for a given educational experience.</i> "	Commission
200527	7.01	Added components related to credit hour policies and "... comparable for all methods of educational delivery (i.e., distance education)."	Commission
200527	7.02	certificate in CHM requires 210 hrs clinical training with 200 hrs of herbal internship	Commission
200527	7.05	Modified clinic observation requirements	Commission
200527	7.06	Added " <i>credit hours granted</i> "	Commission
200527	9.10	Added program component reference to " <i>program delivery</i> "	Commission
200527	10.02	Removed " <i>for listed degrees, the degree granting institution must be listed</i> "	Executive Director
191113	5.02	Removed speaking score requirements from " <i>PHASED CHANGE TO ENGLISH LANGUAGE PROFICIENCY REQUIREMENTS</i> "	Commission
190104	1.03	Added DAOM programmatic components	Commission
190104	5.02	Added DAOM programmatic components	Commission
190104	5.02	Added native-English countries to education option for language proficiency	Executive Director
190104	5.03	Added DAOM programmatic components	Commission
190104	7.02	Added DAOM programmatic components	Commission
190104	7.04	Added DAOM programmatic components	Commission
190104	7.05	Added DAOM programmatic components	Commission
190104	7.08	New criterion added to Comprehensive Standards and Criteria	Commission
190104	8.02	Added DAOM programmatic components	Commission
181206	7.02	Added add'l detail re: clinical training hours allocation for PD programs	Executive Director
181206	9.05	Added Legal and Reg Checklist as a linked reference	Executive Director
181206	Stand. 6 criteria	Adjusted use of "assessment" and "evaluation" to align with ACAOM Glossary definitions	Executive Director
181020	5.08	Added corrective language re: foreign training evaluation	Executive Director
181020	6.04	Specified applicability to master's and PD programs	Executive Director
181020	Stand. 8 criteria	Updated to incorporate Guest Lecturers	Commission
181020	10.01	Updated to incorporate Guest Lecturers; clarify published materials to include electronic media	Commission
181020	10.02	Updated to incorporate Guest Lecturers; clarify published materials to include electronic media	Commission
180811	4.03	Added privacy requirements for non-Title IV institutions	Commission
180811	4.04	Added patient privacy requirements for non-Title IV institutions	Commission
180811	5.02	Added Chinese herbology cert. programmatic components	Commission
180811	7.02	Added Chinese herbology cert. programmatic components	Commission
180811	7.04	Added Chinese herbology cert. programmatic components	Commission
180811	7.05	Added Chinese herbology cert. programmatic components	Commission

180811	Legal & Reg Checklist	Added info re: human subjects research	Commission
180611	6.04	Added links to performance data thresholds	Executive Director
180507	Legal & Reg Checklist	Initial public posting date	Executive Director
180507	All Criteria	Initial public posting date	Executive Director